



Tort Law (9th edn)

Kirsty Horsey and Erika Rackley

p. 101 5. Special duty problems: Psychiatric harm

Kirsty Horsey and Erika Rackley

<https://doi.org/10.1093/he/9780198925231.003.0005>

Published in print: 15 August 2025

Published online: August 2025

Abstract

This chapter begins by explaining the meaning of psychiatric harm. Though initially psychiatric harm was recoverable only if accompanied by physical injury, it is now clear that the claimant can recover for pure psychiatric harm so long as it is a recognised psychiatric illness. The chapter considers the distinction between ‘primary’ and ‘secondary’ victims and other circumstances where the law recognises victims of psychiatric harms as having a claim in negligence (rescuers, involuntary participants, communicators of shocking news, self-harm by the defendant and ‘assumption of responsibility’ cases).

Keywords: Keywords, psychiatric injury, negligence, primary victim, secondary victim, duty of care

Problem question

Read this problem question carefully and keep it in mind while you are working through the chapter that follows. At the end of the chapter you will be able to apply what you have learnt to the problem question and advise the relevant parties.

Following months of speculation the legendary indie guitar band *Blinking Idiot* are about to embark on a reunion tour of the UK. They are performing a warm-up gig at a small intimate venue when a spotlight falls onto the stage, causing a massive explosion, killing the band members: Zoe, Amish and Ennio. Unfortunately, the lighting rig (onto which the spotlight was fitted) had been negligently maintained by Rack & Horse Lighting. The sight is particularly gruesome.

Hua, Amish's wife, is watching the gig from the VIP area of the venue. She is physically unharmed, but later suffers nightmares and depression. This is particularly traumatic for her as she had previously suffered from depression but had sought help and recovered.

Lex, Zoe's brother, is listening to the live radio broadcast of the gig from his hotel room in Paris. He hears the explosion and thinks he can hear Zoe screaming. He rushes to the airport, managing to catch a flight that is just leaving, and arrives at the hospital three hours after the accident. Unfortunately, Zoe's body has not yet been moved to the morgue and is still covered in blood and grime from the explosion. He develops post-traumatic shock disorder.

Inez has attended every *Blinking Idiot* gig in the UK and has travelled to a number of their overseas concerts. She is a founder member of their fan club and regularly contributes to their fan magazine. She always tries to stand as close as possible to the stage. Miraculously she was not hurt by the explosion but has since been overcome with grief.

Tao was one of the first on the scene. They are a trainee paramedic and this was their first major accident. They rush to the stage but quickly see that there is little they can do. They spend the next two hours comforting distraught fans. They later suffer from recurring nightmares and panic attacks.

Kaz, one of the roadies, is overcome with feelings of guilt and depression. It was his job to fix the lighting and he feels the explosion was his fault. A subsequent investigation completely exonerates him.

Advise the parties.

p. 102 5.1 Introduction

Consider the following examples:

→ *A grandfather watches as his grandchild is hit and killed by a car driven by a drunk driver. He later develops severe depression.*

→ *A teenage boy is involved in a car accident. Though he escapes physical injury, he suffers a reoccurrence of anxiety and depression.*

→ *An elderly woman is trapped in a poorly serviced lift for 12 hours. During the experience she is very frightened and afterwards suffers from claustrophobia and insomnia.*

→ *A doctor misdiagnoses a child's heart condition. They later suffer a heart attack while out shopping with their aunt. The aunt later develops PTSD.*

→

The organisers of a gig negligently let too many people into the venue. The gig is being broadcast live and a young teenager sees his boyfriend caught in the crush at the front of the stage. Six hours later his mother tells him that his boyfriend has died. He becomes increasingly withdrawn and is unable to sleep.

In all these cases, the nature of the harm suffered is not physical but mental or psychological. The distinction between physical and purely psychiatric injuries is an important one.¹ The courts have been far more cautious in recognising claims in respect of psychiatric harms than they have in relation to physical injuries, developing a restrictive body of rules severely limiting the circumstances in which victims may recover compensation for their mental injuries. Whether this distinction can be justified as a matter of principle or policy—and indeed whether there is in fact any clear and coherent distinction between physical and psychiatric harms—is far from certain.

5.2 What is psychiatric harm?

Psychiatric harm is a form of personal injury. When the courts first started considering claims in respect of psychiatric injuries, they focused on the way in which such harm tends to be caused: that is, by an ‘assault’ on an individual’s mind or senses rather than a physical impact on the claimant’s body. This explains older references to this area of law as relating to liability for ‘nervous shock’. Psychiatric harms were also grouped together with certain physical harms caused by what one has witnessed. However, the distinct policy issues raised by psychiatric injuries have seen the focus shift away from the way the harm is caused to the type of harm suffered. As David Ibbetson notes:

Before the middle of the twentieth century the courts took a restrictive attitude towards liability for ‘nervous shock’; the focus here was not on the type of injury but on the way in which it had been caused. Gradually this twisted round until ‘nervous shock’ was identified with ‘post-traumatic stress disorder’—a type of harm rather than a mode of causation of harm—from which it shifted yet further into ‘psychiatric injury’.²

p. 103 ↪ Nowadays the courts maintain a clear distinction between physical and pure psychiatric harm, with different rules applying to each. This is notwithstanding developing medical knowledge and increased awareness of the serious and incapacitating nature of mental illnesses and the difficulties in categorising injuries as either physical or psychiatric. Despite judges acknowledging that this distinction is largely unreal and unsustainable,³ there remains a general reluctance to let go of the assumption that mental harm—particularly when it is caused as a result of concern for another’s safety—should be treated differently to physical harm.

Initially psychiatric harm was recoverable only if accompanied by physical injury—hence, for example, a number of the early cases involving the claimant suffering a miscarriage or premature birth following a traumatic experience.⁴ However, it is now clear that claimants can recover for what is described as ‘pure’ psychiatric injury (that is, where it is only injury caused) so long as the injury caused is a recognised

psychiatric illness. As Lord Steyn noted in *White v Chief Constable of South Yorkshire Police* [1998], ‘the law cannot compensate for all emotional suffering even if it is acute and truly debilitating’ (at 491).⁵ It is not possible to recover in the tort of negligence for ‘mere’ grief, anxiety or distress (*Hicks v Chief Constable of South Yorkshire Police* [1992]; *Brock v Northampton General Hospital NHS Trust* [2014]).⁶ While some conditions such as depression, schizophrenia, adjustment disorders, post-traumatic stress disorder (PTSD) and anxiety neurosis are clearly recognisable as psychiatric illnesses, it can be difficult to distinguish ‘ordinary’ feelings of anxiety or distress from an actionable harm and so it may be that the question is one of degree, rather than type, of harm.⁷

Pause for reflection

Consider for a moment the facts of *Hicks*. The teenage claimants, Sarah and Victoria Hicks, were crushed and suffocated to death in the Hillsborough Stadium disaster (discussed in **section 5.6**). The House of Lords rejected a claim for psychiatric harm brought on their behalf (by their father, Trevor Hicks) holding that the police’s negligence had *only* caused the Hicks sisters to suffer ‘distress’, which was not recoverable. The evidence suggested that they had become unconscious within seconds and died within five minutes. Do you agree with the House of Lords’ decision? At what point, if any, should negligently inflicted ‘distress’ ground a claim for psychiatric injury? Why do you think Trevor Hicks brought this claim? Think again about the purposes of tort law discussed in **Chapter 1**.

Test your understanding by trying this section’s self-test questions <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-5-2>.

p. 104 5.3 The general exclusionary rule

In the early 1990s, the courts developed a number of control mechanisms in order to restrict recovery for negligently inflicted psychiatric harm. Opinions differ as to the extent to which these mechanisms are problematic. While some see them as necessary in order to protect defendants from crushing liability, others suggest that they represent a combination of invidious distinctions and convoluted, often contradictory rules grounded in shallow and inadequate theoretical foundations.⁸ Why is it, for example, that a house owner was able to recover in respect of psychiatric harm caused by witnessing a fire which extensively damaged her home (*Attia v British Gas* [1988]) while a brother who saw his sibling being crushed to death was unable to claim (*Alcock v Chief Constable of South Yorkshire Police* [1992])? Similarly, why was a mother who saw her family in hospital a few hours after a tragic road accident (*McLoughlin v O’Brian* [1982]) able to recover but not a father who watched his newborn son die over the course of five days as a result of medical negligence (*King v Royal United Hospitals Bath NHS Foundation Trust* [2021]) or a mother who encountered her daughter’s body a few moments after her death which had been caused by a doctor’s negligent failure to diagnose and

treat her severe pneumonia three days earlier (*Paul and another v Wolverhampton NHS Trust; Polmear and another v Royal Cornwall Hospitals NHS Trust; Purchase v Ahmed* [2024])? And why allow recovery to a social worker whose traumatic caseload caused them to have a nervous breakdown (*Walker v Northumberland County Council* [1995]) but not to the traumatised police officers involved in the Hillsborough Stadium disaster (*White*)?

It may be, as Conaghan and Mansell suggest, that ‘all of these outcomes can be explained in terms of the web of rules which have been spun round cases of psychiatric harm but that does not make them any more defensible when placed side by side and considered in terms of justice and basic common sense’.⁹ Without doubt, the application and the development of these control mechanisms has been somewhat haphazard. The courts have largely acted without clear policy goals, with the result that their reasoning and, on occasions, decisions have been motivated both towards and away from allowing recovery according to the—typically tragic—circumstances of particular cases.

Arguments for restricting compensation for pure psychiatric harm

The key arguments for restricting compensation in relation to negligently inflicted psychiatric harm were summarised by Lord Steyn in *White*.

- (1) The difficulties of drawing a line between acute grief and psychiatric illness and the greater diagnostic uncertainty in relation to psychiatric claims (and the related fear of sham claims and doubts as to causation).
- (2) The effect the increased availability of compensation might have on potential claimants, particularly as a disincentive to rehabilitation.
- (3) ↵ The significant increase in the class of claimants who could recover and the alleged danger of an over-proliferation of claims (the ‘floodgates’ argument).
- (4) The potential unfairness to the defendant of imposing damages out of all proportion to the negligent conduct (including the increased burden on insurers and, ultimately, all insurance policy holders) (at 493–4).

Counterpoint

As long ago as 1998, the Law Commission in its *Report on Liability for Psychiatric Illness*¹⁰ concluded that most of the arguments for restricting liability for negligently inflicted psychiatric harm do not stand up to close scrutiny.¹¹ Not least because many apply equally well to claims for physical injury. Consider, for example, the momentarily careless car driver—the absence of proportionality between culpability and consequences in this situation is as prominent and problematic whether a claim following an accident is for physical or psychiatric injury (although, of course, it is important to note

that, unlike psychiatric harm, physical injuries are likely to be limited to those in physical proximity to the accident). Moreover, despite perhaps greater distrust (at least by some) in relation to psychiatric illnesses, fraudulent or exaggerated claims are just as likely in relation to physical harm. Although medically it can be determined that an injury has occurred, it often cannot establish to any degree of certainty the extent or duration of the pain and suffering it has caused. A classic example is a neck injury (whiplash) usually following a car accident. There is no way of ‘proving’ the pain the claimant is in—the courts have to go on their word.¹²

Test your understanding by trying this section’s self-test questions <<https://iws.oup.support.com/embed/access/content/horse9e-student-resources/horse9e-self-test-questions-5-3>>.

5.4 ‘Primary’ and ‘secondary’ victims

Typically when discussing claims in respect of psychiatric harm a distinction is drawn between ‘primary’ and ‘secondary’ victims. Different rules apply to each. The distinction reflects different ways that psychiatric injuries may be suffered. Very broadly, a claimant may suffer such injuries **either** as a result of fearing for their own safety, i.e. that something does, or is about to, happen to *them* (e.g. being involved in a car accident) **or** as a result of seeing or hearing about something that has happened to *someone else* (e.g. witnessing a car crash involving other people). While the former will be considered a primary victim, the latter falls within the category of secondary victim.¹³ The traditional distinction between primary and secondary victims can be summarised in Table 5.1.

Table 5.1 The traditional distinction between primary and secondary victims

Primary victims	Secondary victims
Claimant must suffer a recognised psychiatric injury (McLoughlin)	Claimant must suffer a recognised psychiatric injury (McLoughlin)
Physical injury must be foreseeable; however, psychiatric injury itself need not be foreseeable. No need for injury to be reasonably foreseeable in claimant of ‘ordinary fortitude’ (Page) unless claimant exposed to risk of stress in the workplace (Barber) or the event has not occurred (Rothwell)	Psychiatric injury must be foreseeable in a person of ‘ordinary fortitude’ in the same circumstances as the claimant
Application of Page —is the claimant in the ‘zone of physical danger’? Note: unwitting agent ‘exception’—where primary victim is ‘a participant who believes ‘has or is about to be involuntary cause of another’s death’ (Dooley)	Psychiatric injury must be caused by an ‘accident’ or external event (Paul)

Primary victims	Secondary victims
	Application of Alcock control mechanisms: close tie of love and affection, proximity to the accident and means by which the shock is caused
No policy considerations to limit the number of claimants	Policy used to limit the number of claimants

p. 106 ↪ Though the language of primary and secondary victims is usually traced to Lord Oliver’s opinion in **Alcock**, the factual distinction between those who suffer psychiatric injury as a result of being involved in an accident and those who suffer psychiatric injury through witnessing an accident involving others can be found in the earlier case of **Bourhill v Young** [1943], discussed in section 5.6. **Alcock** was one of a number of cases arising out of the Hillsborough Stadium disaster.¹⁴ Faced with a tragedy involving multiple victims and a huge number of potential claimants, Lord Oliver drew a distinction between cases in which the claimant is ‘involved, either mediately or immediately [that is, indirectly or directly], as a participant’—primary victims—and those in which the claimant was ‘no more than the passive and unwilling witness of injury caused to others’ (at 407). In relation to the latter group of claimants—secondary victims—the House of Lords went on to articulate a number of limits on the availability of claims (detailing what constitutes a relationship of sufficient proximity between the claimant and defendant—described by Henry LJ in the Court of Appeal as the ‘nearness, heariness and dearness’ requirements (*Frost v Chief Constable of South Yorkshire Police* [1997] at 278¹⁵)).

Since **Alcock**, the distinction between primary and secondary victims has been applied in differing ways. While Lord Oliver in **Alcock** had clearly envisaged the primary victim category to encompass a fairly wide class of claimants, including rescuers and unwitting agents, p. 107 ↪ Lord Lloyd in **Page v Smith** [1996] appeared to restrict it to those within the range of foreseeable physical danger—an approach which was utilised to strategic effect by the majority of the House of Lords in another Hillsborough-related case, **White**. More recently, the Supreme Court in **Paul** put an end to what had been a gradual expansion of the secondary victim category, by ruling out the vast majority of claims brought by secondary victims in the context of medical negligence.

At other times, the courts have allowed claims without considering whether the claimant is either a ‘primary’ or ‘secondary’ victim (though the label is sometimes applied later). In **Yearworth v North Bristol NHS Trust** [2009], for example, the Court of Appeal allowed the claims of claimants who had banked sperm with the hospital prior to embarking on treatment for cancer and who had suffered psychiatric harm suffered as a result of the defendant’s negligent destruction of their samples without considering whether they were primary or secondary victims.¹⁶

Similarly, there are a number of cases where a claimant has been able to recover (without establishing their primary/secondary victim status) on the basis that the defendant has ‘assumed responsibility’ to ensure that they avoid reasonably foreseeable psychiatric injury.¹⁷ These include **Waters v Commissioner of Police for the**

Metropolis [2000] (employer and employee); *Calvert v William Hill Credit Ltd* [2008] (bookmaker and gambler); *AB v Leeds Teaching Hospital NHS Trust* [2005] (doctor and patient); *Swinney v Chief Constable of Northumbria Police* [1997] (police and police informant); and *Butchart v Home Office* [2006] (prison officer and prisoners). In the latter case, the claimant (who the prison authorities knew to be in a depressed and suicidal state) was housed with another suicidal prisoner, who subsequently went on to commit suicide. The claimant woke up to find his cell mate had hanged himself. He suffered severe shock. He was later told that his cell mate's suicide was his fault. The Court of Appeal held that prison authorities owed the claimant a duty of care to ensure the health and safety of prisoners and that this could extend to preventing or minimising the risk of a vulnerable prisoner claimant suffering psychiatric harm as a result of being placed in a cell with a suicidal prisoner.

This category also includes occupational stress claims. As with the other assumption of responsibility cases, these do not usually involve the apprehension of physical impact (or danger); rather their psychiatric injury typically stems from unreasonable direct pressure or stress they feel as employees. An employer was first found liable for their employee's work-related stress in *Walker v Northumberland County Council* [1995], in which Colman J held that 'there was no logical reason why risk of psychiatric damage should be excluded from the scope of an employer's duty of care' (at 710). The claimant, a social services manager with a heavy and emotionally demanding caseload of child abuse cases, had suffered a second nervous breakdown as a result of pressure at work (after his earlier breakdown, he had been promised additional support, which had not materialised). *Walker* was confirmed by the House of Lords in *Barber v Somerset County Council* [2004].

p. 108 The claimant, a teacher who had not been appropriately supported on his return to work after ↵ having been signed off previously for stress and depression (caused by the level of work expected of him), suffered a mental breakdown. Although they came to a different decision on the facts—allowing the claimant's appeal—the law lords approved the 'practical propositions' set out by Hale LJ (later Lady Hale) in the Court of Appeal (where the case was reported as *Hatton v Sutherland* [2002]): the 'threshold question', in line with the ordinary principles of employer's liability, was whether the kind of harm to the particular employee was (or ought to have been) reasonably foreseeable. Foreseeability depends on the interrelationship between the individual characteristics of the relevant employee and the requirements made of them by their employer, including (but not limited to) the nature and extent of the work being undertaken, signs of stress shown by the employee themselves, the size and scope of the business and the availability of resources. And once the threshold is crossed, it is immaterial whether a person of ordinary fortitude would have suffered the same harm.

Counterpoint

The lack of precision as to the definition of a primary—or (less often) a secondary—victim is problematic.¹⁸ As Lord Leggatt and Lady Rose note in *Paul*, '[p]recisely how the distinction is or should be drawn, if at all, potentially raises difficult questions' (at [51]). However, while we may consider the distinction unsatisfactory, if we are to continue to distinguish between primary and secondary victims (and, at least for the time being, this appears to be the case), it is important that we define these terms with a measure of certainty. And we should be wary of extending them (and their

accompanying rules) to cases they don't fit and of using them to justify limitations on recovery that are unnecessarily restrictive. After all, as Lord Phillips CJ noted in *French v Chief Constable of Sussex Police* [2006], 'there is no magic in this terminology' (at [31]). With this in mind, while it is important to understand the various uses of the 'primary' and 'secondary' victim classifications as different rules apply to them, the terms are, at best, a means to the end of determining what the claimant needs to establish in order to succeed in their claim.

Test your understanding by trying this section's self-test questions <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-5-4>.

5.5 Primary victims

It has long been established that a claimant is able to recover damages for psychiatric injury stemming from actual physical injury or from a reasonable fear or apprehension of danger to their physical safety (*Dulieu v White* [1901]).¹⁹ This was expanded in *Page v Smith*.

p. 109

***Page v Smith* [1996] AC 155 (HL)**

The claimant's car was involved in a minor road traffic accident caused by the defendant's negligence. Although the claimant suffered no physical injury, the accident triggered the recurrence of a condition which causes severe fatigue and which had been in remission at the time of the accident.²⁰ The claimant argued that this had become chronic and permanent as a result of the accident.

Although initially successful, the claimant lost in the Court of Appeal on the ground that his injury was not reasonably foreseeable in a person of ordinary courage and fortitude. A bare majority of the House of Lords allowed his appeal. In his leading opinion, Lord Lloyd held that where it is reasonably foreseeable that the defendant's negligence may cause *physical* harm to the claimant they can also recover for the *psychiatric* harm they suffer, even where no physical injury actually occurred:

Suppose, in the present case, the plaintiff had been accompanied by his wife, just recovering from a depressive illness, and that she had suffered a cracked rib, followed by an onset of psychiatric illness. Clearly, she would have recovered damages, including damages for her illness, since it is conceded that the defendant owed the occupants of the car a duty not to cause physical harm. Why should it be necessary to ask a different question, or apply a different test, in the case of the plaintiff? Why should it make any difference that the physical illness that the plaintiff undoubtedly suffered as a result of the accident operated through the medium of the mind, or of the nervous system, without physical injury? If he had suffered a heart attack, it cannot be doubted that he would have recovered damages for pain and suffering, even though he suffered no broken bones. It would have been no answer that he had a weak heart. (Lord Lloyd at 187)

Adopting Tobriner J's description in the US case *Dillon v Legg* (1968) of the claimant having been in 'the zone of physical danger' (at 80), Lord Lloyd held that Page was able to recover for the psychiatric harm he suffered notwithstanding the fact that (a) he had suffered no physical harm and (b) the psychiatric injury itself was not reasonably foreseeable. On this basis, as long as some form of physical injury was foreseeable, it does not matter whether it was foreseeable that the claimant's particular *psychiatric* illness would have occurred in a person of ordinary moral courage and fortitude.²¹ In cases where *physical* harm is reasonably foreseeable (i.e. where the claimant is in the 'zone of physical danger'), physical and psychiatric harms are to be treated as equivalent, so that if the former is foreseeable, the claimant can recover in respect of both physical *and* psychiatric harms, even in cases where the latter is *not* foreseeable.

p. 110 ↵ The decision in *Page* has been highly criticised.²² Academic criticism has tended to focus on Lord Lloyd's restrictive definition of a primary victim as a party who is *necessarily* within the zone of physical danger.²³ Opinions differ over whether Lord Lloyd intended to limit the primary victim category to those claimants who were in physical danger, and it remains unclear as to when a claimant *who is not in peril* will be recognised as a primary victim.²⁴ This was considered by the House of Lords in *Rothwell v Chemical and Insulating Co Ltd* [2007].

Rothwell v Chemical & Insulating Co Ltd and another; Topping v Benchtown Ltd (formerly Jones Bros Preston Ltd); Johnston v NEI International Combustion Ltd; Grieves v FT Everard & Sons Ltd and another [2007] UKHL 39 (HL)

The claimants had been exposed to asbestos in the course of their employment and had developed pleural plaques (areas of tissue that have become thickened in the lining of the lung). While the pleural plaques themselves would not have developed into a more serious condition (if they had, the claimant would have been able to recover), they were evidence that asbestos had entered the claimants' bodies and showed that the claimants, therefore, might in the future develop a more serious asbestos-related disease. The claimants were physically healthy, but were claiming on the basis that they were worried they may develop a serious condition in the future on the basis and that since it was reasonably foreseeable that the defendant's negligence put them at risk of physical injury (of contracting asbestosis or mesothelioma) they were primary victims and hence should be owed a duty of care in respect of their psychiatric illness on the basis of *Page*.

The claims failed. Neither the pleural plaques themselves nor the claimants' anxiety (or in one case psychiatric illness) that they might become seriously ill was enough to ground their claims. Mere anxiety about an increase of a risk of developing a disease in the future is not actionable in tort. '[T]he category of primary victim,' Lord Hope held, 'should be confined to persons who suffer psychiatric injury caused by fear or distress resulting from involvement in an accident caused by the defendant's negligence or its immediate aftermath' (at [54]). Psychiatric injury suffered as a result of a fear that something that might happen in the future fell within 'an entirely different category' (at [54]) and as such was—and should continue to be—unrecoverable. Moreover, Lord Hoffmann continued,

The general rule still requires one to decide whether it was reasonably foreseeable that the event which actually happened (in this case, the creation of a risk of an asbestos-related disease) would cause psychiatric illness to a person of reasonable fortitude. (at [30])

Thus while it was expected that developing pleural plaques would cause a person of reasonable fortitude some anxiety, a more serious or significant reaction (such as the development of a recognised psychiatric illness as suffered by one of the claimants) was unforeseeable.

Pause for reflection

The requirement in *Rothwell* that the claimant's reaction needs to be reasonably foreseeable in a person of reasonable fortitude appears to be at odds with the approach to foreseeability in *Page*. This point was made by counsel for one of the claimants. In response, Lord Hoffmann, declining the opportunity to depart completely from *Page*, sought to confine the reasoning in *Page* to cases where the claimant's psychiatric injury is the result of being in the vicinity of an event (accident) that has *actually occurred* (at [32]). 'It would,' he argued, 'be an unwarranted *extension* of the principle in *Page v Smith* to apply it to psychiatric illness caused by apprehension of the possibility of an unfavourable event which had not actually happened' (at [33], emphasis added).

On this basis, *Rothwell* and *Page* can be distinguished. *Page* applies where the event has actually taken place, while the requirement that psychiatric harm be reasonably foreseeable in a person of ordinary fortitude (as set out in *Rothwell*) is only necessary where the claimant's psychiatric illness has been caused by the apprehension, or increased risk, that an event may occur in the future.

Quite why it was necessary to refine *Page* in this way is unclear. Lord Hope's reference to Lord Steyn's opinion in *White* in which he cautions against judicial expansion in relation to recovery for negligently inflicted psychiatric harm, and his own view that 'the labels ... identified in *Page v Smith* should not be extended beyond what was in contemplation in that case' (at [54]), may provide an explanation. It is likely Lord Hope was mindful of the extent of liability should the claimant succeed.²⁵

But what about claimants who suffer psychiatric injury as a result of believing—rightly or wrongly—that they have caused injury to another? This category of claimants are known as involuntary participants or 'unwitting agents'. The key case here is *Dooley v Cammell Laird & Co Ltd* [1951], which predates—and has survived—the leading case law in this area.

***Dooley v Cammell Laird & Co Ltd* [1951] 1 Lloyd's Rep 271 (Liverpool As-sizes)**

The claimant was operating a crane at the docks where he worked when, as a result of his employer's negligence, the sling connecting the load to the crane-hooks snapped causing the load to fall into the hold of a ship where men were working.

He successfully recovered from the psychiatric illness he suffered as a result of his fear that the falling load would injure or kill some of his fellow workmen.

p. 112 ↪ Though Lord Oliver held that *Dooley* had been correctly decided in *Alcock*, there was some doubt as to whether it survived the reining in of the primary victim category in *Page* and the ‘thus far and no further’ sentiment of Lord Steyn in *White* (after all, Dooley was never himself in the zone of physical danger).²⁶ Nevertheless, a number of more recent cases factually analogous to *Dooley* have sidestepped the requirement of physical imperilment, preferring the view of Lord Oliver in *Alcock*—where his status as a ‘primary victim’ arose out of his role as a ‘participant’ in the accident—that he ‘had been put into the position of being or believing that he is, has been, or is about to be the involuntary cause of another’s death or injury’ (at 408), rather than as a result of being in physical danger. In *Monk v PC Harrington Ltd* [2008], for example, the claimant was a foreman working on a construction site at Wembley Stadium. A temporary working platform became dislodged (as a result of the defendant’s negligence) and fell 60 feet and hit two workmen below, killing one and injuring the other. The foreman heard of the accident on a portable radio and went to help the injured workmen. He subsequently developed significant psychiatric injury which prevented him from working. The court held that though the primary victim category extended to ‘unwilling participants’, and that this extended to those who reasonably felt that they had put another in danger, on the facts there was no reasonable basis for the claimant’s belief that he was responsible for his co-worker’s death.²⁷

One final point on primary victims. Following *Paul*, the distinction between whether a claimant’s psychiatric injury was caused by an ‘accident’ or ‘incident’ has become important in the context of claims by secondary victims in clinical settings. However—at least for the time being—this is not the case in relation to primary victims. And so where a mother sustains psychiatric injury due to negligence during the management of labour and before the child is born, she is able to recover as a primary victim (see e.g. *Wild v Southend University Hospital NHS Foundation Trust* [2014]; *Wells and Smith v University Hospital Southampton NHS Foundation Trust* [2015]; *RE (A minor by her mother and Litigation Friend LE) and others v Calderdale & Huddersfield NHS Foundation Trust* [2017]; and *YAH v Medway NHS Trust* [2018]).²⁸

Test your understanding by trying this section’s self-test questions <<https://iws.oup.support.com/embed/access/content/horse9e-student-resources/horse9e-self-test-questions-5-5>>.

5.6 Secondary victims

p. 113 ↪ A secondary victim suffers psychiatric injury as a result of witnessing someone else being harmed or endangered.²⁹ In such cases it doesn’t matter if the primary victim, that is the person the claimant thought was in danger, is not killed or injured at all. The claim of the secondary victim is not ‘parasitic’ upon a claim by the primary victim.³⁰ However, recovery in secondary victim cases is limited by a number of policy-orientated control mechanisms. The first of these is that the psychiatric injury suffered must be reasonably foreseeable in a person of ‘ordinary fortitude’ in the same circumstances.³¹ The notion of ordinary phlegm or fortitude, derived from *Bourhill v Young* [1943], is invoked as a means of assessing the ‘validity’ of a claimant’s emotional reactions in the face of trauma.

***Bourhill v Young* [1943] AC 92 (HL)**

The claimant, who was eight months pregnant, witnessed a serious motorbike accident. Though she did not see the accident itself, she heard it, and saw the motorcyclist's blood on the pavement. She later suffered serious psychiatric harm and her child was stillborn about a month after the accident. The House of Lords rejected her claim on the grounds that her injuries were not foreseeable—she was never in physical danger—and that, as a pregnant woman, she was particularly susceptible to shock.

If, therefore, a secondary victim suffers psychiatric harm in circumstances where the ordinarily courageous person would not, the defendant will not be liable even if a severe psychological reaction results. However, once some psychiatric harm is foreseeable, the defendant will—on the basis of the 'egg shell' or 'thin' skull rule—be liable in full, even if a particular vulnerability or susceptibility means that the claimant suffers much greater psychiatric harm than might have been anticipated (*Brice v Brown* [1984]).

Pause for reflection

It has been argued that the notion of 'ordinary phlegm or fortitude' not only allows for the incorporation of evaluative judgements (and possible gender bias) in the consideration of what is a 'normal' reaction to any given event but also perpetuates the false assumption that there is (or can be) a reasonable response to a tragic event. Do you agree? Consider, again, the facts of *Bourhill*. To what extent are the characteristics of the claimant significant here? What degree of fortitude is to be expected of an 'ordinary pregnant woman'? Or is a pregnant woman, by definition, not 'ordinary'?

p. 114 **5.6.1 The evolution of claims for psychiatric harm: *McLoughlin, Alcock* and *White***

The foundations for the modern approach to secondary victims were laid down in a trilogy of cases in the 1990s: *McLoughlin v O'Brian*, *Alcock v Chief Constable of South Yorkshire Police* [1992] and *White v Chief Constable of South Yorkshire Police* [1999] (also known as *Frost*).

***McLoughlin v O'Brian* [1982] 1 AC 410 (HL)**

The claimant suffered psychiatric harm after witnessing the 'immediate aftermath' of a serious car accident, which killed her daughter and injured her husband and three other children. Arriving at the hospital some two hours after the accident, she encountered circumstances that were 'distressing in

the extreme and ... capable of producing an effect going well beyond that of grief and sorrow' (at 417). Her husband and children, visibly upset and bruised, were still covered in grime and dirt from the accident.

The House of Lords allowed her claim on the basis that she had come upon 'the immediate aftermath' of the accident. However, their lordships were not in full agreement as to *why* she should be able to recover. In particular, there was tension as to whether the issue of recovery should be determined, as Lord Bridge suggested, on the ordinary principles of *reasonable foreseeability* or, as Lord Wilberforce argued, by reference to independent *policy-based* factors—including the (a) *closeness of the relationship* between the claimant and the accident victim, (b) the *proximity* of the claimant to the accident itself and (c) whether the shock was induced by what the claimant *saw or experienced* as opposed to what she was *told* after the event (at 422–3).³²

Even at the time *McLoughlin* was viewed as a borderline case—on 'the margin of what the process of logical progression would allow' (Lord Wilberforce at 419). Nevertheless, it encapsulated the judicial understanding of the time; a time when judges were more generous to claimants in negligence than they are today. However, darker clouds were building on the horizon, as the mood within tort law generally shifted towards a more restrictive approach to the duty of care (the so-called 'retreat from *Anns*').³³ Then the unthinkable

p. 115 ↪ happened, a tragedy with multiple victims and a vast number of potential claimants and this mood was cemented—the Hillsborough Stadium disaster.³⁴

The Hillsborough Stadium disaster

'The tragedy that claimed the lives of 96 Liverpool football fans shattered a community and shook the world of football took a matter of minutes to unfold. In its simplest terms, Hillsborough was a case of overcrowding in the central standing area allocated to Liverpool fans at the FA Cup semi-final match against Nottingham Forest ... The disaster began to unfold at approximately 2.30pm. With half an hour before kick-off, most of the Nottingham Forest supporters were in their seats. Meanwhile the area reserved for the Liverpool supporters—the Leppings Lane end of the stadium—was half empty. But outside, it was a different story, with more than 2,000 Liverpool supporters building up against the turnstiles to the Leppings Lane entrance. Some had arrived late from their journey across the Pennines. Others had stayed outside to make the most of the sunshine. There were also those who had come without tickets to the all-ticket match, hoping to buy them at the ground. But whatever the reason for the late rush, anxiety among both fans and police was mounting as the minutes to kick-off ticked by.'

By 2.45pm the crowd had swelled to over 5,000, making entry to turnstiles virtually impossible. Those who did get through were short of breath and sweating profusely from the crush. As the minutes passed, it became increasingly clear that, despite police efforts, the mass of people would never get through by 3pm. There was also a more serious risk of some being dangerously hurt. Something had to be done.

Suddenly at 2.52pm, the large blue, concertina steel door—Gate C—in the perimeter wall was slid open by a police officer. Those at its entrance tumbled through. Those at the back pushed harder still. The logjam was unstuck. But things quickly got out of control. Where fans had been entering in ones and twos through the turnstiles, there was now a wave of about 2,000 racing to see the start of the game. The majority took the most obvious route: straight ahead through the tunnel of gangway 2. They piled into the back of pens 3 and 4, which were already uncomfortably full, crushing those at the front.

At 2.54pm the teams came onto the pitch. Fans at the back of the pens pushed forward for a better view, unaware that people were dying in the front. As the excitement of the game grew, there were more surges, each causing a squeeze more perilous than the last. Finally, with fans spilling through a narrow escape onto the pitch or being lifted to the seating areas above, a policeman realised what was happening. At 3.06pm, six minutes into the game, he ordered the referee to stop the game. Only then did the scale of the disaster become clear. Bodies were lifted forward and laid out on the pitch—many teenagers and children. People screamed for their loved ones as ambulance staff fought to save lives. Advertising hoardings were torn down as makeshift stretchers in a desperate attempt to bring faster relief.

p. 116

↵ By 4.50pm, the scheduled end of the game, the ground was empty. Abandoned clothing and programmes littered the scene of the disaster. While nearby the bodies of the dead lay in the stadium's gymnasium.'

(BBC News 'Timetable to a tragedy' 14 April 1999 referring to Taylor LJ's vivid account in *Interim Report on the Hillsborough Stadium Disaster* (Cm 765, 1989))

You may find it interesting to watch the *Guardian* report 'Hillsborough: anatomy of a disaster' where journalist David Conn brings together footage from the day to explore what happened on 15 April 1989 and why it has taken so long for victim-survivors and their families to get answers. It is 11 minutes long so if you don't have time to watch it now, make a note and come back to it later.

We understand that the images and discussion of events in the video might be upsetting and that it can be hard to watch. It is not essential that you watch this film. If you need to, please do stop and take a break and talk to someone if that would be helpful.

Ninety-seven people were crushed to death and over 400 people were injured at Hillsborough Stadium. At the time, it was ‘one of the most televised, monitored and photographed disasters in the UK’.³⁵ Many thousands of people watched the tragedy unfold on live TV or listened to it on the radio, even more saw later news reports and press coverage. The number of potential claims for psychiatric harm was immense. The court’s response, in the second and third cases in the trilogy, was in the words of Lord Hoffmann, to abandon its aspiration to ‘provide a comprehensive system of corrective justice ... in favour of cautious pragmatism’ (*White* at 502).

***Alcock v Chief Constable of South Yorkshire Police* [1992] 1 AC 310 (HL)**

Alcock was a test case involving representatives of the friends and families of the victims of the disaster at Hillsborough Stadium. The claimants included grandparents, friends, spouses, siblings and fiancés of the victims who had either been at the ground when the incident occurred, or who had arrived at the ground later or seen or heard about it on the TV or radio. As they had not been *directly* involved in the disaster (in the sense of having been in physical danger), the claimants were not able to recover as primary victims. Rather, they argued that their psychiatric injuries were caused by what they saw happen to and/or feared had happened to their loved ones and that, as such, they were secondary victims. The defendant, the Chief Constable of South Yorkshire Police, argued that he did not owe a duty of care to those who had suffered psychiatric damage as a result of seeing or hearing the news of what had happened.³⁶

p. 117

↪ Despite limited success at first instance, both the Court of Appeal and House of Lords rejected the claims. The claimants were not ‘in contemplation of law, in a relationship of sufficient proximity to or directness with the tortfeasor [the chief constable] so as to give rise to a duty of care’ (at 410). Drawing on and developing the more restrictive approach of Lord Wilberforce in *McLoughlin*, Lord Oliver set out guidelines—sometimes referred to as the *Alcock* control mechanisms—as to when proximity will be established:

first, that in each case there was a marital or parental relationship between the plaintiff and the primary victim; **secondly**, that the injury for which damages were claimed arose from the sudden and unexpected shock to the plaintiff's nervous system; **thirdly**, that the plaintiff in each case was either personally present at the scene of the accident or was in the more or less immediate vicinity and witnessed the aftermath shortly afterwards; and, **fourthly**, that the injury suffered arose from witnessing the death of, extreme danger to, or injury and discomfort suffered by the primary victim. **Lastly**, in each case there was not only an element of physical proximity to the event but a close temporal connection between the event and the plaintiff's perception of it combined with a close relationship of affection between the plaintiff and the primary victim. It must, I think, be from these elements that the essential requirement of proximity is to be deduced, to which has to be added the reasonable foreseeability on the part of the defendant that in that combination of circumstances there was a real risk of injury of the type sustained by the particular plaintiff as a result of his or her concern for the primary victim. (at 411, emphasis added)

Lord Oliver was not intending to set down rigid rules for recovery—he saw no logic or policy reason, for example, for limiting claims to parents or those who were married or for precluding claims where there was a sufficiently 'close tie of affection'. However, even in *Alcock* they appeared to be treated as such: two of the claimants (who had been present at the ground and watched the tragedy at Hillsborough unfold), one of whom lost two brothers in the disaster and the other who lost his brother-in-law, were unable to recover as they had not been able to demonstrate a sufficiently close tie of love and affection.

The claims of other family members who had not been at the ground but arrived around eight hours later and who saw their family members' bodies in the temporary mortuary that had been set up in the stadium's gymnasium, failed because they were deemed to have arrived too late to fall within the scope of the 'immediate aftermath' of the disaster, while those who had watched the accident on television and who (in line with editorial policy) had not been able to identify individual primary victims were held not to be sufficiently close to the accident or its aftermath to be treated as being at the stadium.

The final case in the original trilogy was *White v Chief Constable of South Yorkshire Police*. This case was brought by police officers who had been on duty at Hillsborough and who had suffered psychiatric injury (mostly PTSD) as a result of their actions that day. Three were on duty at the ground itself, one had attempted to free spectators, two had attended the makeshift morgue in the gymnasium and another two were among those drafted in later that afternoon and who, with the others, witnessed the chaotic and gruesome scenes. A final officer had worked as a liaison officer at a nearby hospital.

p. 118 ↩ Controversially, in the Court of Appeal, the claims of all but one of the police officers had been successful on the basis that they were rescuers and/or employees. Many believed it was unfair to allow the police (who in another capacity were the defendants in *Alcock* and *White*) to recover when the claims of

friends and family had been dismissed in *Alcock*.³⁷ As Lord Steyn noted in *White*: ‘The claim of the police officers on our sympathy, and the justice of the case, is great but not as great as that of others to whom the law denies redress’ (at 498). By the time the police officers’ case reached the House of Lords, it was clear that the law in this area was in a ‘genuine doctrinal muddle’.³⁸ The Law Commission, just one year earlier, had recommended limited statutory reform to remove the proximity and direct perception requirements of the *Alcock* criteria.³⁹ The common law position was so far beyond judicial repair that Lord Steyn concluded that:

the only sensible general strategy for the courts is to say thus far and no further ... to treat the pragmatic categories as reflected in the authoritative decisions such as the *Alcock* case ... and *Page v Smith* as settled for the time being (*White* at 500).

Any further development, the law lords concluded, was the responsibility of Parliament.

***White v Chief Constable of South Yorkshire Police* [1998] 2 AC 455 (HL)**

The police officers argued that they were entitled to recover on the basis that they were either rescuers or employees.

A bare majority of the House of Lords rejected these arguments. Accepting the narrow definition of primary victim given by Lord Lloyd in *Page*, Lord Steyn in the majority held that a rescuer could only be considered a primary victim if he ‘objectively exposed himself to danger or reasonably believed he was doing so’ (at 499). To extend the category of primary victims to include rescuers would be, in Lord Steyn’s view, ‘unwarranted’ (at 500). As the police officers were never in (nor did they reasonably believe themselves to be in) actual physical danger, they could not recover on this basis.

Nor could they recover on the basis that the defendant owed them a duty of care because they were his employees. Though employers clearly owed a duty of care to their employees in respect of physical injuries, the majority held that it did not follow that there was *also* a duty in respect of psychiatric harms suffered in the course of their employment.⁴⁰ To allow the claimants to recover on the basis of their employment with the defendant would lead to ‘striking anomalies’: the police officers would be given rights denied to others, including doctors and ambulance workers, who had assisted the injured at Hillsborough out of a sense of moral, as opposed to legal, obligation (Lord Hoffmann at 506).

As they were neither primary victims, nor to be treated as falling within a category outside this categorisation, the claimants needed to satisfy the requirements for a claim by secondary victims as set out in *Alcock* and summarised by Lord Hoffmann as follows:

(1) The plaintiff must have close ties of love and affection with the victim. Such ties may be presumed in some cases (e.g. spouses, parent and child) but must otherwise be established by evidence. (2) The plaintiff must have been present at the accident or its immediate aftermath. (3) The psychiatric injury must have been caused by direct perception of the accident or its immediate aftermath and not upon hearing about it from someone else. (at 502)

And, as they could not do so, their claims failed.

Without doubt, the law lords' aim in *White* was one of damage limitation. One casualty of this approach was the treatment of rescuers.

Traditionally, the courts treated rescuers favourably: 'Danger invites rescue. The cry of distress is the summons of relief ... the act, whether impulsive or deliberate, is the child of the occasion' (*Wagner v International Railway Co* [1921]). So, where a defendant endangered another by their carelessness, the courts typically held that it is reasonably foreseeable that others may put themselves at risk by trying to save the victim. As such, defendants have been held to owe a duty of care not just to those they initially endanger by their actions, but also to those who intervene to rescue them (see e.g. *Ogwo v Taylor* [1988] and *Baker v TE Hopkins* [1959]).⁴¹

In *Chadwick v British Railways Board* [1967] this approach was extended to include rescuers who suffered psychiatric harm as a result of their actions thereby allowing the widow of a window cleaner to recover for the psychiatric harm suffered by her husband as a result of his particularly harrowing and gruesome experience while giving help and relief to victims of a severe rail crash over the course of 12 hours.

And yet, while Lord Oliver in *Alcock* clearly envisaged rescuers falling with the category of primary in line with the 'well-established' principle that a defendant owes a duty of care to those 'induced to go to the rescue' of those in peril as a result of their negligence (at 408), this long-standing inclination to protect rescuers was jettisoned in favour of what Lord Hoffmann described as 'a practical attempt, under adverse conditions, to preserve the general perception of the law as a system of rules which is fair between one citizen and another' (*White* at 511).

There does not seem to me to be any logical reason why the normal treatment of rescuers on the issues of foreseeability and causation should lead to the conclusion that, for the purpose of liability for psychiatric injury, they should be given special treatment as primary victims when they were not within the range of foreseeable physical injury ... such an extension would be unacceptable to the ordinary person ... He would think it wrong that policemen, even as part of a general class of persons who rendered assistance, should have the right to compensation for psychiatric injury out of public funds while the bereaved relatives are sent away with nothing. (Lord Hoffmann in *White* at 510)

p. 120

And so following *White* the court's reasoning in *Chadwick* has been reframed and its decision justified (*Chadwick* has not been overruled) on the basis that Mr Chadwick was within the zone of physical danger—and as such a primary victim (even though he did not realise this himself and this was not part of the original court's reasoning)—when he sought to extract the victims from the wreckage of the train. It is clear. however, that today a rescuer is only able to recover if they are (or at least reasonably believed themselves to be) in foreseeable risk of physical injury—that is, if they meet the primary victim requirements set out in *Page*—or if they meet the secondary victim requirements set out in *Alcock*.

Counterpoint

Lord Goff (who would have allowed the police officers' claims on the basis that they were rescuers), in his dissenting opinion in *White*, rejects this reinterpretation of *Chadwick* and criticises the introduction of further 'control mechanisms' (akin to the highly criticised *Alcock* restrictions) through the creation of 'unacceptable' and 'unjust' distinctions, which distinguish claimants according to their physical location (at 487). He argues that the majority's approach in making foreseeability of physical injury a *necessary* condition of liability for psychiatric harm (at 479) was not only inappropriately restrictive but also in opposition to Lord Lloyd's expansive strategy in *Page* and Lord Oliver's categorisation of rescuers as primary victims in *Alcock*. The imposition of a requirement of fear of physical injury in such cases is, he argues, both capricious and misplaced. Drawing on *Chadwick*, and responding to the majority's attempt to distinguish it, he poses the following analogy:

Suppose that there was a terrible train crash and that there were two Chadwick brothers living nearby, both of them small and agile window cleaners distinguished by their courage and humanity. Mr A Chadwick worked on the front half of the train, and Mr B Chadwick on the rear half. It so happened that, although there was some physical danger present in the front half of the train, there was none in the rear. Both worked for 12 hours or so bringing aid and comfort to the victims. Both suffered PTSD in consequence of the general horror of the situation. On the new control mechanism now proposed, Mr A would recover but Mr B would not. To make things worse, the same conclusion must follow even if Mr A was unaware of the existence of the physical danger present in his half of the train. This is surely unacceptable. (at 487–8)⁴²

Which view do you find most persuasive?

5.6.2 Application of the *Alcock* criteria

Described by Tomlinson LJ as ‘both arbitrary and pragmatic’ in *Liverpool Women’s NHS Foundation Trust v Ronayne* [2015] (at [11]), it is, nonetheless, worth looking at the so-called *Alcock* ‘control mechanisms’ in a little more detail—noting again that in all instances it is necessary for the claimant to establish first that they are suffering from a medically recognised psychiatric illness. In order to be a secondary victim, the claimant must establish:

- (1) a close tie of love and affection with the person killed, injured or imperilled (i.e. the primary victim);
- (2) that they were present at the accident or its immediate aftermath; **and**
- (3) that their psychiatric injury was caused by a direct perception of the accident or its immediate aftermath and not upon hearing about it from someone else.

Pause for reflection

Over the years, the so-called *Alcock* criteria have been the subject of trenchant criticism. Indeed Jonathan Morgan has suggested that ‘no area of English tort law has been more fiercely criticised’.⁴³ Almost immediately, it was suggested that this area of law was one in which the ‘silliest rules prevail’.⁴⁴ The Law Commission’s *Report on Liability for Recovery for Psychiatric Illness*, published in between the House of Lords’ decisions in *Alcock* and *White*, stated that the *Alcock* criteria had ‘been almost universally criticized as arbitrary and unfair’—going on to recommend the statutory removal of all of them but the requirement of a close tie of love and affection.⁴⁵ Even in *Alcock* itself, as Lord Briggs has noted:

some of their lordships expressed disquiet about where the courts had thus far set the boundaries for ... liability [for pure psychiatric harm], and a view that any further development of this area of liability should be undertaken by the legislature.⁴⁶

This was repeated again in *White* where Lord Steyn was of the view that:

the only sensible general strategy for the courts is to say thus far and no further. The only prudent course is to treat the pragmatic categories as reflected in authoritative decisions such as *Alcock* and *Page v Smith* as settled for the time being but by and large to leave any expansion or development in this corner of the law to Parliament. (at 500)

↵ But is the criticism of *Alcock* fair? Certainly, there have been times where the criteria have appeared unduly restrictive. *Alcock* is a case in point of where claims have been denied that might be thought should have succeeded. Take Jane Stapleton’s criticism of the ‘love and affection’ requirement, for example:

That ... claims can turn on the requirement of 'close ties and affection' is guaranteed to produce outrage. Is it not a disreputable sight to see brothers of Hillsborough victims turned away because they had *no more* than brotherly love towards the victim? In future cases will it not be a grotesque sight to see relatives scrabbling to prove their especial love for the deceased in order to win money damages and for the defendant to have to attack that argument?⁴⁷

However, Donal Nolan offers a different view. He suggests the *Alcock* control mechanisms have been used in a way that was unintended by the House of Lords. He argues that the law lords did not seek to lay down rigid rules precisely because, in Lord Jauncey's words, 'to draw such a line would necessarily be arbitrary and lacking in logic' (*Alcock* at 422). Thus, as well as being a landmark case in tort, *Alcock* is, in his view, also a 'misunderstood decision' with the result that:

a case that is often painted as retrograde and reactionary was in fact rather conservative, and even in some respects mildly progressive, not least when one takes into account the circumstances in which it was decided.⁴⁸

Nolan has a point. Over three decades later, perhaps it is time judges stopped blaming *Alcock* and took responsibility for the limitations of their own decisions. Certainly this was the view of Lord Burrows in his dissenting judgment in *Paul*:

There is no need to give up on the incremental development of the common law in this area. Moreover, the incremental development of the law is one that has been explicitly entrusted by the Government to the courts in this area: ... Lord Steyn's approach in *Frost [White]* of 'thus far and no further' is no longer justified if it ever was. (at [203]–[204])

As we shall see, Lord Burrows couldn't persuade his colleagues, but what do you think?

5.6.2.1 A close tie of love and affection

As Lord Oliver made clear in *Alcock*, it is not the case that siblings and other relatives—or even friends—can *never* be secondary victims. Rather, it is simply that they must bring evidence to prove such ties existed. And similarly, while a close tie of love and affection will be presumed in the case of spouses, parents and children, this can be rebutted if the defendant can show that such closeness did not exist. But proving a close tie of love and affection between friends and more distant family members on a case-by-case basis may be easier said than done.

p. 123 ↵ In *Robertson v Forth Road Bridge Joint Board* [1996], for example, the claimant suffered psychiatric harm after his colleague died while trying to remove a sheet of metal from the Forth Road Bridge in windy conditions. His claim was denied despite evidence showing that they had spent the greater part of their

employment working together, and that they had often walked to and from work together and had gone out socially during the week. Though clearly their relationship went beyond that of work colleagues, the court held that it was not enough to demonstrate the necessary close tie of love and affection. And while in *Alcock* Lord Ackner, at least, was not prepared to rule out the possibility of a ‘mere bystander’ being able to recover if the circumstances were ‘particularly horrific’, it is difficult to imagine quite what would be more horrific than the events at the Hillsborough Stadium.⁴⁹ This was considered in *McFarlane v EE Caledonia Ltd* [1994]. The claimant developed PTSD after witnessing the Piper Alpha oil rig disaster from a boat which was engaged in trying to fight the fire which had engulfed the rig. He argued that, as the fire was especially horrific, the oil-rig owners (who had admitted responsibility for the disaster) owed him a duty of care as a bystander to the incident. The Court of Appeal rejected his claim on the basis that he was neither a primary⁵⁰ nor secondary victim (he did not have a sufficiently close tie of love and affection with those on the rig).

What if the claimant has a close tie of love and affection with the primary victim but is unaware that they are in danger? This was the case in *Young v Downey* [2020] where the claimant, who was 4½ years old at the time, sought to recover damages for the psychiatric injury suffered following her father’s death in the Hyde Park bombing in 1982.⁵¹ She had watched her father leaving the barracks and had heard the bomb blast. She had later seen other severely injured soldiers returning to the barracks. In her evidence, she stated: ‘I remember telling my Mum afterwards “Daddy should be coming now” but he never did’. Rejecting her claim for psychiatric harm, Spencer J held, contrary to the evidence of the psychiatric expert,⁵² that her comment ‘Daddy should be coming now’ suggested that she had ‘no appreciation that her father had been involved’ in the bombing (at [29]). He continued:

the need for the claimant to have a close tie of love and affection with the person killed, injured or imperilled incorporates *an appreciation* that the loved one is or may have been the one, or one of those, killed injured or imperilled ... Thus the identification of the loved one as the primary victim is an important, indeed in my judgment, an essential element. ([30]–[31], emphasis added)⁵³

p. 124 5.6.2.2 Presence at the accident or its immediate aftermath

In relation to the second requirement of ‘proximity’—closeness to the accident in time and space—Lord Oliver summed up the general position thus:

The necessary element of proximity between plaintiff and defendant is furnished, at least in part, by both physical and temporal propinquity and also by the sudden and direct visual impression on the plaintiff’s mind of actually witnessing the event or its immediate aftermath. (at 416)

But how far does the immediate aftermath extend? Certainly at times the case law has seemed to point in different directions. Mrs McLoughlin’s arrival at the hospital two hours after the accident was within the ‘immediate aftermath’.⁵⁴ Attempts by the claimants in *Alcock* to extend this failed. The several family members travelled to the ground after the disaster, the first arriving at the scene around eight or nine hours later. While Lord Ackner thought this might be part of the aftermath of the event, it was not part of its *immediate* aftermath (at 405). Nor, in Lord Jauncey’s (much criticised) view, was the purpose of their visit

(which was, sadly, to identify their loved ones' bodies) comparable to that of a loved one arriving at the immediate scene to provide solace and comfort (at 424). Robert Alcock's search of the Hillsborough ground and arrival at the temporary mortuary in the stadium's gymnasium around midnight (about eight hours after the match was abandoned) where he identified his brother-in-law's body which was blue with bruising, his chest red, was not therefore sufficient to establish the necessary proximity to the accident—the blood on his brother-in-law's body being memorably described by Jane Stapleton as 'too dry' to allow recovery.⁵⁵

While there were some signs of extending the immediate aftermath of an accident in *Galli-Atkinson v Seghal* [2003], where the courts held it to include various component parts including a mother's visit to the scene of the accident and later seeing her daughter's body at the hospital morgue, on the whole courts have adopted a strict approach to determining the 'immediate aftermath' of an accident.⁵⁶ The Supreme Court in *Paul* confirmed that 'immediate aftermath' should be understood within the confines of *McLoughlin* and that the approach in *Galli-Atkinson* should not be followed (at [122]):

a reasonably clear line can be drawn if heed is paid to the observations of Lord Wilberforce that allowing the claim in *McLoughlin* (a) was 'upon the margin of what the process of logical progression would allow' ... and (b) depended critically on the evidence that, when the claimant came upon the members of her family, 'they were in the same condition [as they had been at the roadside], covered with oil and mud, and distraught with pain'. (Lord Leggatt and Lady Rose at [108])

p. 125 ↪ In *Taylor v A Novo (UK) Ltd* [2013] the claimant sought to avoid the limitations of the proximity requirement by arguing that there were, in fact, two events—the latter of which was the 'operative' or 'qualifying' event.

***Taylor v A Novo (UK) Ltd* [2013]**

The claimant developed PTSD as a result of witnessing her mother's sudden death three weeks after she had been negligently injured at work. In bringing her claim against her mother's employers, the claimant sought to establish proximity by arguing that there were, in fact, two 'events'—the accident (at which the claimant had not been present) and her mother's subsequent collapse at home and death (which she had witnessed)—and that the cause of her injuries was the latter. This analysis was rejected by the Court of Appeal.

In reality there was a single accident or event ... which had two consequences. The first was the injuries to [the mother's] head and arm; and the second (three weeks later) was her death ... if [the claimant] had been in physical proximity to her mother at the time of the accident and had suffered shock and psychiatric illness as a result of seeing the accident and the injuries sustained by her mother, she would have qualified as a secondary victim on established principles. But in my view, to allow [her] to recover as a secondary victim on the facts of the present case would be to go too far ... if ... right, [she] would have been able to recover damages for psychiatric illness even if her mother's death had occurred months, and possibly years, after the accident (subject, of course, to proving causation). This suggests that the concept of proximity to a secondary victim cannot reasonably be stretched this far. (Lord Dyson at [29]–[30])

Lord Leggatt and Lady Rose in *Paul* put it somewhat more bluntly:

The reason why the claim in *Novo* failed was that, although there was an external, traumatic, event (ie 'an accident') which immediately caused injury to Mrs Taylor, the claimant did not witness that event and the event which she did witness and which caused her psychiatric illness was not an accident. (at [90])⁵⁷

p. 126 5.6.2.3 Direct perception of the accident or its immediate aftermath

The House of Lords in *Alcock* was clear that there would be no liability to a secondary victim who is merely told about the shocking event by a third party—for example, through newspaper coverage or television broadcasts:

Although the television pictures certainly gave rise to feelings of the deepest anxiety and distress, in the circumstances of this case the simultaneous television broadcasts of what occurred cannot be equated with the 'sight or hearing of the event or its immediate aftermath.' Accordingly shocks sustained by reason of these broadcasts cannot found a claim. (Lord Ackner at 405)

However, once again, the law lords in *Alcock* did not rule out recovery for anyone watching TV images of a shocking and distressing event in principle. Lord Ackner left open the possibility that watching a live broadcast could exceptionally ground a claim if it is clear that the victims had died—referring to Nolan LJ's example in the Court of Appeal of a hot-air balloon carrying a number of children exploding live on television. (at 405)

Pause for reflection

While this might be the most straightforward of the three *Alcock* control mechanisms, it may also be that the reasoning underpinning it has been superseded by changes in technology. Were Nolan LJ's exploding hot-air balloon example to happen today, it is very likely that the Broadcasting Codes of ethics and practices would prevent the broadcasting of the distressing and shocking image. It is, however, just as likely that unedited and distressing images of the event would quickly find their way onto online and social media platforms. Here the potential for indeterminate liability seems greater than ever. It is not clear that Lord Ackner's reasoning as to the lack of equivalence between these images and actually being at the event would hold.

Of course, this does not mean that *only* the original tortfeasor should be liable (if at all). The uploaded images might be treated as a *novus actus interveniens*, extending an argument made in *Alcock*,⁵⁸ thus breaking the chain of causation and limiting the defendant's liability. But should anyone else be liable? What about the website host or social media platforms? This public interest in the dissemination of information might well be taken to preclude liability for the negligent communication of distressing news. However, while we may well feel that there should be media freedom to convey important information to the public, even if some find it shocking, and that we should not allow people to recover simply on the basis that they have been upset by what they have seen or heard, it is not clear that this should automatically or necessarily extend to those who facilitate the sensationalisation or misrepresentation of such information, particularly where it causes viewers distress amounting to psychiatric injury. Where would you draw the line?

p. 127 5.6.3 The impact of *Paul v Wolverhampton NHS Trust; Polmear and another v Royal Cornwall Hospitals NHS Trust; Purchase v Ahmed*

Most of the cases discussed so far in this chapter have involved some kind of accident—an external, unexpected and unintended event, a road or rail crash, for example, or an accident at a building site or workplace or large-scale disasters such as the Hillsborough Stadium disaster. However, alongside these another line of case law has been developing that seeks to establish whether the same *Alcock* conditions might be applied to cases of psychiatric harm caused in a clinical or medical context. And so, in 2024, the trilogy of leading cases in this area was joined by *Paul and another v Wolverhampton NHS Trust; Polmear and another v Royal Cornwall Hospitals NHS Trust; Purchase v Ahmed*.

Before learning more about *Paul*, you might like to watch our video 'Reorientating the patchwork quilt: over to you, Lord Burrows' <<https://iws.oup.support.com/video/access/content/horsey9e-student-resources/horsey9e-video-5-reorientating-the-patchwork-quilt-over-to-you-lord-burrows>>, recorded

before the Supreme Court handed down its decision in January 2024, where we explore the legacy of the Hillsborough Stadium disaster on the ability of secondary victims to recover for psychiatric harm and the glimmer of hope that was offered by *Paul*.

Paul is the latest in a line of—largely unsuccessful—case law, spanning a 22-year period, arising out of claims by family members seeking to recover for psychiatric injuries caused by witnessing the traumatic death or treatment (or immediate aftermath thereof) of a loved one as a result of the defendant's negligent medical treatment and/or misdiagnosis. Over the years, the vast majority of claims have failed; however, one exception was *North Glamorgan NHS Trust v Walters* [2002].⁵⁹ In this case, a mother was able to recover as a secondary victim for the psychiatric harm she suffered as a result of the negligent treatment leading to the death of her baby son. The court held that the necessary shocking event was not confined to a single moment in time and, taking a realistic approach to the facts, the 36-hour period prior to her son's death could be classed as a single horrifying event. Referring to Lord Wilberforce's opinion in *McLoughlin*, Ward LJ continued:

One looks to the totality of the circumstances which bring the claimant into proximity in both time and space to the accident. It seems to me, therefore, to be implicit in [Lord Wilberforce's] judgment read as a whole that when he said ... 'the shock must come through the sight or hearing of the "event" or its "immediate aftermath"' he was not intending to confine 'the event' to a frozen moment in time. (at [27])

Nevertheless, over the following two decades, similar claims were denied either on the basis that the claimant had not encountered the 'immediate aftermath' (*Taylor v Somerset Health Authority* [1993]), because there was no 'sudden shock' (*Sion v Hampstead Health Authority* [1994]) and on the basis that the claimants were not able to identify a single horrifying event (*Shorter v Surrey & Sussex Healthcare NHS Trust* [2015]; *Liverpool Women's Hospital NHS Foundation v Ronayne* [2015]).

Walters was beginning to look increasingly out of line and in *Paul* the Supreme Court held that it had been wrongly decided and should not be followed⁶⁰ (at [121]).

p. 128

***Paul and another v Wolverhampton NHS Trust; Polmear and another v Royal Cornwall Hospitals NHS Trust; Purchase v Ahmed* [2024] UKSC 1 (SC)**

Three cases were heard together. In each case, the claimants had suffered psychiatric injury after witnessing, or coming upon the immediate aftermath of, the sudden and unexpected death of a parent or child. In each case, the death had been caused by negligent (or lack of) treatment by a medical professional which had occurred variously over a year, six months or just three days previously. In all

cases, but for the negligence, each of the primary victims would have survived. The question for the court was whether an individual can claim for psychiatric injury caused by witnessing the death or other horrifying event of a close relative as a result of earlier clinical negligence.

In *Paul*, the claimants' father died suddenly of a heart attack while the family were out shopping.

His daughters saw him fall backwards and hit his head on the pavement. They tried to call their mother on their mobile phones and to call an ambulance, which was eventually called by a passer-by. When their mother arrived, the daughters were taken to a nearby church. They heard their mother screaming their father's name. They came out and saw an ambulance crew put a foil blanket over their father and paramedics performing chest compressions on him. Mr Paul was taken by ambulance to hospital but was declared dead on arrival. (at [9])

In *Polmear*, the claimants' 7-year-old daughter, Esme, died of pulmonary veno-occlusive disease (a rare condition that causes high blood pressure in the lungs). Her father had met Esme after she had felt unwell on a school trip.

[Her father] left her at the door of the school but shortly afterwards was called back and found her lying on the floor with a member of staff administering first aid. He took over and tried to give Esme mouth-to-mouth resuscitation. She was not breathing. Esme's mother ran to the school and saw her lying on the floor with members of staff attempting resuscitation which she could see was not working ... Both parents went with Esme in an ambulance to hospital where she was declared dead. (at [13])

In *Purchase*, the claimant's daughter, Evelyn, died at home of severe pneumonia. The claimant arrived home to find her daughter 'lying motionless on her bed with the house telephone in her hand, staring at the ceiling and not moving'. She later realised that:

she had a missed call and a voice message from Evelyn on her mobile phone. The voice message was the sound of Evelyn's dying breaths which continued for four minutes and 37 seconds. The call was timed at 4.40 am and ended approximately five minutes before [she] got home and saw Evelyn. (at [17])

All the claims had failed in the Court of Appeal on the basis that, in the words of Vos MR:

Novo is binding authority for the proposition that no claim can be brought in respect of psychiatric injury caused by a separate horrific event removed in time from the original negligence, accident or a first horrific event ... even where a horrific event is the first occasion on which any damage is caused to the primary victim'. (at [96]–[97])

↩ However, Vos MR continued:

If I were starting with a clean sheet, I can quite see why secondary victims in these cases [i.e. involving clinical negligence] ought to be seen to be sufficiently proximate to the defendants to be allowed to recover damages for their psychiatric injury. Since, however, this court is bound by *Novo*, it is for the Supreme Court to decide whether to depart from the law as stated by Lord Dyson in that case. (at [12])

The Supreme Court disagreed. Dismissing the appeals by a majority of 6:1, Lord Leggatt and Lady Rose concluded:

We are not able to accept that the responsibilities of a medical practitioner, and the purposes for which care is provided, extend to protecting members of the patient's close family from exposure to the traumatic experience of witnessing the death or manifestation of disease or injury in their relative. To impose such a responsibility on hospitals and doctors would go beyond what, in the current state of our society, is reasonably regarded as the nature and scope of their role ... the persons whom doctors ought reasonably to have in contemplation when directing their minds to the care of a patient do not include members of the patient's close family who might be psychologically affected by witnessing the effects of a disease which the doctor ought to have diagnosed and treated. Hence there does not exist the proximity in the relationship between the parties necessary to give rise to a duty of care. (at [138], [142])

'No analogy could reasonably be drawn,' they continued:

between the situation with which *McLoughlin*, *Alcock* and *Frost* [*White*] were concerned where illness is caused by witnessing an accident (or its immediate aftermath) involving a victim with whom the claimant has a close tie of love and affection and situations where the claimant does not witness an accident but suffers illness as a result of witnessing such a person suffering a medical crisis. (at [115])

Refusing to overrule *Novo*, Lord Leggatt and Lady Rose argued that:

to extend the scope of allowable claims by secondary victims to situations where the claimant witnesses the death or illness of a relative from disease would give rise to unacceptable and unfair differences in treatment between different categories of claimant. It would be impossible to explain to an ordinary reasonable person why, for example, damages can be recovered by a daughter who sees her parent die from a heart attack or pulmonary embolism which should have been avoided, but compensation is denied to, say, a mother who did not witness a road accident in which her child was fatally injured or its 'immediate aftermath' but identifies the mutilated body afterwards in the mortuary or is present at the hospital when her child dies many days later from injuries sustained in the accident. Such unjust differences in outcome could, of course, be avoided by removing the second and third requirements for claims by secondary victims established by the House of Lords in *Alcock* and confirmed in *Frost*. But it would not, in our view, be right to contemplate such a radical departure from settled law. (at [115]–[116])

p. 130

Lord Burrows dissented. He would have allowed recovery in each of the cases before the court. Doing so, he argued, was justified on two grounds. Either there was nothing in the earlier case *↗* law requiring the claimant's psychiatric injury to be caused by an 'accident' (in the sense of something external and violent happening to the primary victim) or—even if there was—then allowing the appeals would be justified as a legitimate incremental extension of the common law (at [198]–[204]). As it is, he continued:

Although by insisting on an accident one might be said to be applying the same legal principles to medical negligence as to other areas of negligence causing psychiatric illness to secondary victims (for example, negligent driving), adoption of that approach ... would in practice result in a blocking off of medical negligence as an area where, subject to rare exceptions, there can be no liability for psychiatric illness suffered by secondary victims. To block off this area of negligence is not an attractive approach where the Government has explicitly entrusted flexible development of the law to the courts. (at [207]–[208])

Without doubt, *Paul* is the most significant case on psychiatric injury since *Alcock*. It was heard by a panel of seven judges. This unusually large panel was convened as it was initially thought that the Justices might be asked to depart from their decisions in *McLoughlin*, *Alcock* and *White* (at [50]). This was not the case. Rather, the decision in *Paul* reaffirms the primacy of the *Alcock* criteria in cases involving secondary victims and provides important clarifications where the majority felt the law had taken a 'wrong turn' (at [78]).

5.6.3.1 Need for an accident + *Alcock* criteria

The starting point for the majority's reasoning was that allowing recovery for secondary victims in certain limited circumstances is an exception to the general common law position, statutory interventions aside, denying recovery to a claimant for the death or personal injury caused by the defendant to someone else (at [2]).⁶¹ Lord Leggatt and Lady Rose frame the issue as being about whether:

this exceptional category of case includes—or can and should be extended to include—cases where the claimant's injury is caused by witnessing the death or injury of a close relative, not in an accident, but from a medical condition which the defendant has negligently failed to diagnose. (at [5])

In other words, what principally required justification was not, in the majority's view, the limits that have been placed on the ability of secondary victims to recover, but rather the fact that they can recover at all. For this, no positive justification was advanced. Lord Leggatt and Lady Rose instead relied on the view of Lord Oliver in *Alcock* that 'the existence of this exception as "now too well established to be called in question"' (at [140]). But framing it in this way allowed Lord Leggatt and Lady Rose to accept the 'rough and ready logic' provided by the approach in *Alcock*:

in limiting recovery by secondary victims to individuals who were present at the scene, witnessed the accident and have a close tie of love and affection with the primary victim. These

↪ limitations are justified, not by any theory that illness induced by direct perception is more inherently worthy of compensation than illness induced by other means; but rather by the need to restrict the class of eligible claimants to those who are most closely and directly connected to the accident which the defendant has negligently caused and to apply restrictions which are reasonably straightforward, certain and comprehensible to the ordinary person. (at [141])

Lord Leggatt and Lady Rose go on to draw a vital distinction between cases involving 'accidents' where relatives or loved ones of the primary victim go on to develop psychiatric harm versus other occasions, for example a medical 'incident', 'condition' or 'crisis', where people develop psychiatric harm as a result of a tragedy befalling their loved one as a result of somebody else's negligence. This distinction between an 'accident'—that is, 'an unexpected and unintended event which caused injury (or the risk of injury) to a victim by violent external means' (at [24])—and a 'medical crisis' is central to their reasoning (at [53]).⁶² While recovery by secondary victims is possible in relation to the former, it is not in relation to the latter—for, as noted earlier, 'no analogy can be drawn' between the two (at [115]):

We have not been asked on these appeals to alter or abrogate the limits on the recovery of damages by secondary victims in accident cases established by the decision of the House of Lords in *Alcock*. Instead, this court is asked to recognise as analogous a category of cases in which illness is sustained by a secondary victim as a result of witnessing a death or manifestation of injury which is not caused by an external, traumatic event in the nature of an accident but is the result of a pre-existing injury or disease. For the reasons given, we do not consider that such cases are analogous. (at [142])

Lord Leggatt and Lady Rose give three reasons to justify this distinction. First, ‘an accident is, by definition, a discrete event in the ordinary sense of that word, meaning something which happens at a particular time, at a particular place, in a particular way’, allowing for there to be a level of certainty (at [108]). Secondly, and more controversially, witnessing an accident is ‘likely to be a disturbing and upsetting event’ (at [109]) in contrast to witnessing the injury or illness of a close family member, which while it may be ‘traumatic is also entirely variable’ (at [113]):

It is easy to appreciate the psychological trauma caused to a mother who sees her child run down by a car or to a husband who comes upon the badly injured body of his wife immediately after an accident. Most people would, we think, accept that, if a line is going to be drawn between cases where illness consequent on bereavement is compensable and cases where it is not, distinguishing between claimants who suffered the ordeal of actually witnessing the accident in which a close relative was killed and those who did not is an intelligible place to draw it. (at [109])

Finally (appearing to ignore the application of ‘zone of danger’ test established by *Page*) they drew attention to the difficulty in distinguishing between primary and secondary victims in accident cases—that is whether the psychiatric injury was caused by the claimant’s fear for their own safety or that of others (at [110]).

p. 132 ↪ So where does this leave us? To be clear, following *Paul* it is still potentially possible for a duty to arise to a secondary victim in a medical context, but only if they witness a medical ‘accident’ as opposed to a medical ‘crisis’ (if this is the case then the usual *Alcock* principles apply). This is where future argument and cases will come. Lord Leggatt and Lady Rose left open the possibility of whether it would ever be possible to recover as a secondary victim in a medical setting—for example, if a family member witnessed an acute adverse reaction after a doctor injected a patient with the wrong dose or drug (at [123]). As noted by James Lee, nor do Lord Leggatt and Lady Rose address whether surgery might be characterised as ‘an external traumatic event’ or whether the door has been left open for secondary victim claims for psychiatric injuries caused by witnessing negligence during a particularly distressing labour and birth (referring to *Tredget v Bexley Health Authority* [1994]—the status of which was left undecided by the majority).⁶³

5.6.3.2 No need for a ‘sudden shock’ or ‘horrifying event’

One way in which the courts had sought to limit secondary victim claims brought in a medical context was by emphasising the requirement that the psychiatric harm suffered by the secondary victim must be sustained as a result of what Lord Ackner in *Alcock* described as ‘sudden appreciation by sight or sound of a horrifying event, which violently agitates the mind’ (at 401). This ruled out cases where the injury was caused, for example, by watching the gradual deterioration of a loved one in hospital—such as in *Sion* where a father developed a psychiatric illness as a result of sitting by his son’s hospital bedside for 14 days before his death which was caused by the doctors’ negligent failure to diagnose substantial and continuing bleeding in his son’s kidney following a motorbike accident. It also prevented recovery in cases where there was no single horrifying event, such as in *Shorter* in which the claimant’s sister died from a subarachnoid haemorrhage which had been negligently diagnosed. Rejecting the claim, the court ruled that there had been no ‘seamless single horrifying event’ but rather a series of events relating to her sister’s condition none of which would be

seen as particularly ‘horrificing’ or sudden or unexpected by a person of ordinary fortitude. Similarly, in *Ronayne*, for example, the claimant was unable to recover for psychiatric injuries sustained as a result of watching his wife’s rapid deterioration over the course of 36 hours after undergoing a hysterectomy at the defendant’s hospital and in particular seeing her connected to various machines, including drips and monitors, and her appearance which he later described as resembling the ‘Michelin Man’.⁶⁴ (It was common ground that this was as a result of the defendant’s negligence.) Rejecting his claim, Tomlinson LJ was of the view that:

p. 133

What is required in order to found liability is something which is exceptional in nature ... There is I think a danger of the ‘Michelin Man’ epithet acquiring a significance greater than it ↵ deserves ... I can readily accept that the appearance of Mrs Ronayne on this occasion must have been both alarming and distressing to the claimant, but it was not in context exceptional and it was not I think horrificing in the sense in which that word has been used in the authorities. Certainly however it did not lead to a sudden violent agitation of the mind, because the claimant was prepared to witness a person in a desperate condition and was moreover already extremely angry. (at [41])

Lord Leggatt and Lady Rose make it clear in *Paul* that:

[i]n engrafting onto those requirements additional requirements of needing to prove that the claimant’s injury was caused by the mechanism of a ‘sudden shock to the nervous system’ and was a sufficiently ‘horrificing event’, the law has in our opinion taken an unfortunate wrong turn. (at [78])

There is no need therefore for the event to be sudden or particularly horrificing—not least because there is ‘no available Richter scale of horror’ (at [76]). On this basis, *Sion*, *Shorter* and *Ronayne* had been correctly decided but for incorrect reasons. The claims should have failed simply on the basis that the claimant did not witness the accident (or its aftermath) caused by the defendant’s negligence (at [122]).

Nor need the event be close in time to the negligent act or omission. While typically in accidents the defendant’s negligence and accident happen at more or less the same time, there is no reason, the majority also confirmed, why a gap in time—whether short or long—between the negligence and the accident should affect the defendant’s liability.

Sir Geoffrey Vos MR [in *Novo*] postulated a case of a negligent architect who designs a door in a load-bearing wall without specifying an RSJ, causing masonry to fall on a primary victim’s head years later (paras 79–80). These facts are similar to those of the actual case of *Clay v AJ Crump & Sons Ltd* [1964] 1 QB 533, where an architect who was responsible for the safety of a building site negligently left a wall standing when a building was demolished. The architect was held liable to compensate a person working on the site who was injured when over two months later the wall collapsed. ... we see no reason why, in a case of this kind, the gap in time between the negligence and the accident should prevent a claim by a secondary victim when it does not prevent a claim by a primary victim. If, for example, a mother who was present and saw masonry fall on her child’s head suffered psychiatric injury, her ability to make a claim cannot rationally depend on the length of time between the negligence and the accident. (at [94])

5.6.3.3 Parliament, the NHS and Lord Burrows' dissent

Without doubt, the decision in *Paul* came as a surprise to many. It has been described by Jonathan Morgan as 'a leading case that never was'.⁶⁵ Others have criticised inconsistencies in the majority's reasoning⁶⁶ and drawn attention to the real-life impact of the decision:

The court reasoned it would go far beyond the scope of the responsibility assumed if doctors had to protect patients' friends and relations against the patient's upsetting illness. Witnessing a relative's death was not 'an insult to health from which we expect doctors to take care to protect us but a vicissitude of life which is part of the human condition' (at [139]). Yet must watching one's child or father die *as a result of medical negligence* be shrugged off as mere 'vicissitude'?⁶⁷

p. 134 ↵ Certainly, as Lord Briggs has noted, it is likely that the Court of Appeal and many others were hoping for something more 'along Lord Burrows' reasoning'. But what did Lord Burrows say?

Unsurprisingly given his role at the Law Commission, Lord Burrows was much more critical of the current state of the law than the majority had been. He starts his judgment with the recognition that:

[i]n the context of secondary victims—who can be regarded, generally speaking, as those who suffer psychiatric illness as a result of another's death or injury or imperilment—the tort of negligence draws distinctions that are difficult to defend.

He continues:

It is arguable that the only truly principled solution, which would avoid any arbitrary line drawing, would be to impose a duty of care where, in general terms, it was reasonably foreseeable that psychiatric illness would be caused to the secondary victim (as a person of reasonable fortitude). But policy concerns, in particular the fear of opening the floodgates of litigation, mean that adoption of that principled solution would constitute a radical and giant leap forward for the common law. For that reason, at the present stage of development, that solution is not a realistic option open to the courts. (at [144])

What is open to the courts, in Lord Burrows' view, is a 'justified incremental step ... that has been explicitly entrusted by the Government to the courts in this area' (at [202], [204]). He goes on to set out how he would have allowed the claims to proceed on the basis that 'foreseeability and the control or proximity [to what he considers to be the relevant event—that is the death of their loved one] factors are all satisfied' (at [244]). Rejecting Lord Dyson's reasoning in *Novo* (which he would have overruled), Lord Burrows suggests that in each case (and cases like them) it is the death of the primary victim that is the relevant event, 'not least because it was witnessing the death or its immediate aftermath that caused the psychiatric illness to the secondary victims':

it was reasonably foreseeable (treating the secondary victims as having reasonable fortitude) that they would suffer psychiatric illness as a consequence of the death. It then follows that there should be liability in these three cases because it is not in dispute that, once one treats the event as the death, all the established proximity or control factors [set out in *Alcock*] are satisfied. (at [199])

On this view, the application of the *Alcock* criteria in the context of medical negligence is a justifiable and warranted extension of the common law. In contrast, the approach taken by the majority, and the insistence that there needs to be an accident, is ‘an unwarranted backward step:

Turning the clock back in this way would require, as Lord Leggatt and Lady Rose acknowledge ... the overruling of *Walters* and a departure from the reasoning in almost all of the reported medical negligence cases in this area. Indeed, ... they have left open for another day whether there can be liability even where there has been a medical accident (ie medical negligence comprising or causing an event external to the primary victim ...). In future, and subject to possible rare exceptions, the approach of Lord Leggatt and Lady Rose will mean that recovery for negligently caused psychiatric illness by secondary victims will be closed off in medical negligence cases. (at [250])

p. 135 ↵ However, while Lord Burrows’ reasoning has found favour with clinical negligence lawyers, without doubt had he been able to persuade his fellow Justices the outcome would have been very different for the NHS. In 2023/24, the NHS paid out more than £58,147 million in damages to claimants.⁶⁸ Without doubt, there are difficult choices to be made as to the allocation of such resources. Lord Burrows may be of the view that any increase in the burden on legal liability on the NHS ‘is not the type of socio-economic policy argument that the courts are well-equipped to assess’ and ‘cannot outweigh the reasons of principle and legal policy’ (at [249]). NHS Resolution (the part of the NHS which took *Paul* to the Supreme Court and resolves disputes including following negligence claims) is likely to have breathed a sigh of relief.⁶⁹

Pause for reflection

So where does this leave us?

The law relating to recovery by secondary victims for psychiatric harm has been criticised for well over 25 years now. As long ago as *McLoughlin*, Lord Wilberforce suggested that any substantive change ought only to be made by the legislature. However, in 2009 Parliament made it clear that it was not going to pick up the baton offered to it by both the Law Commission and the law lords in *White*.⁷⁰ As things stand, and as Lord Burrows noted in *Paul*, ‘there is no realistic prospect of legislation’ (at [146]). The issue has been thrown back to the courts to develop—or, as we have seen, not to do so. Instead, after decades of critique the *Alcock* principles have been rehabilitated (with the addition of a novel gloss in medical negligence cases).

This is disappointing. As Lord Briggs has noted, it has taken us over a century to get this far. ‘A man from Mars,’ he continues, ‘would say this was a very slow and haphazard way of carrying out law reform.’⁷¹ We would agree. Do you?

Test your understanding by trying this section’s self-test questions <https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-self-test-questions-5-6>.

5.7 Conclusion

In this chapter we have considered the law in relation to recovery for negligently inflicted psychiatric injury in the *absence* of physical injury. The law in this area has been shaped by prejudice and tragedy. Assumptions that suffering psychiatric harm is less than suffering physical injury and fears of exaggerated and/or fraudulent claims as well as a number of high-profile, negligently caused, disasters have led to the introduction of ‘control mechanisms’ limiting recovery. Central to this is the distinction between claimants deemed ‘primary’ victims, ‘secondary’ victims and, more recently, those who fit neither category. Despite recent clarification offered by the Supreme Court in *Paul*, Lord Steyn’s conclusion in *White* that ‘the law on the recovery of compensation for pure psychiatric harm is a patchwork quilt of distinctions which are difficult to justify’ (at 500) remains apposite.

p. 136 End-of-chapter questions

After reading the chapter carefully, try answering the questions which follow.

- 1 Why does the law distinguish psychiatric from physical injury? To what extent, if at all, is this distinction justifiable?
- 2 What is the distinction between primary and secondary victims and does it produce defensible consequences?
- 3 Does the law relating to psychiatric damage apply coherent principles?

If you would like to know what we think, check the **answer guidance** <https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-5-answers-to-end-of-chapter-questions?options=showName>.

Answering the problem question

Consider again the problem question at the start of this chapter. Now having read about the topic, **what would be your advice to the various parties?**

Here are some pointers to get you started:

- Don't jump straight to the most obvious classification. Though Hua is most likely to be a secondary victim, could she be a primary victim? You should explore what she'd need to establish for this to be the case.
- Will Lex be able to satisfy the proximity in time and space element of **Alcock**? Has he arrived during the 'immediate aftermath' of the explosion?
- It is clear that Kaz is an 'involuntary participant', but could he also be a primary victim? What would he need to establish for this to be the case?

If you need some more guidance:

- See the **annotated version of the problem with issues and cases to consider** <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-5-annotated-problem-question-interactive>>.
- Check your answer against the **suggested outline answer** <<https://iws.oup.support.com/embed/access/content/horsey9e-student-resources/horsey9e-chapter-5-answers-to-problem-question>>.

Further reading

Numerous articles and case notes have been written on the issues surrounding psychiatric damage. The best place to start is Donal Nolan's excellent chapter on **Alcock** or Jonathan Morgan's chapter on 'Psychiatric Illness, Emotional Harm and "Shock"' in his *Great Debates in Tort* book. James Lee's Inner Temple lecture is also well worth watching.

Bailey, Stephen and Donal Nolan 'The *Page v Smith* Saga: A Tale of Inauspicious Origins and Unintended Consequences' [2010] Cambridge Law Journal 495

Briggs, Lord 'Liability for Mental Injury' Chichester University Lecture, 19 January 2024.

Case, Paula 'Now You See It, Now You Don't: Black Letter Reflections on the Legacies of *White v Chief Constable of South Yorkshire Police*' (2010) 18 Tort Law Review 33

Chamallas, Martha and Linda Kerber 'Women, Mothers, and the Law of Fright: A History' (1990) 88 Michigan Law Review 814 ↵

Lee, James 'Incidents and Accidents—The Supreme Court on Psychiatric Harm' Inner Temple Lecture, 6 March 2024: www.youtube.com/watch?v=4t_W3LCWzHM <http://www.youtube.com/watch?v=4t_W3LCWzHM>

5. Special duty problems: Psychiatric harm

Morgan, Jonathan *Great Debates in Tort Law* (Hart 2022) ch 9

Morgan, Jonathan 'Principles, Accidents and Exceptions: Psychiatric Injury in Tort' [2024] Cambridge Law Journal 214

Mulheron, Rachael 'The "Primary Victim" in Psychiatric Illness Claims: Reworking the "Patchwork Quilt"' (2008) 19 Kings Law Journal 81

Nolan, Donal '*Alcock v Chief Constable of South Yorkshire Police* (1991)' in Charles Mitchell and Paul Mitchell (eds) *Landmark Cases in the Law of Tort* (Hart 2010) 273

Teff, Harvey *Causing Psychiatric and Emotional Harm: Reshaping the Boundaries of Legal Liability* (Hart 2008)

Additional suggestions for further reading can be found in this chapter's **web links**. <<https://iws.oup.support.com/ebook/access/content/horsey9e-student-resources/horsey9e-chapter-5-web-links?options=showName>>.

Notes

¹ The focus of this chapter is on claims for negligently caused 'pure' psychiatric injuries, where the *only* injury suffered by the claimant is psychiatric. Where the claimant suffers psychiatric harm as a result of physical injuries negligently inflicted by the defendant, recovery is straightforward (*Corr v IBC Vehicles Ltd* [2008]).

² David Ibbetson *A Historical Introduction to the Law of Obligations* (OUP 1999) 195.

³ See e.g. Kennedy J in *Dulieu v White & Sons* [1901] at 677 and the comment of Lord Lloyd in ***Page v Smith* [1996]**: 'In an age when medical knowledge is expanding fast, and psychiatric knowledge with it, it would not be sensible to commit the law to a distinction between physical and psychiatric injury, which may already seem somewhat artificial, and may soon be altogether outmoded' (at 188).

⁴ See e.g. *Dulieu v White & Sons* [1901]; *Hambrook v Stokes Bros* [1925]; ***Bourhill v Young* [1943]**.

⁵ This was confirmed by the House of Lords in ***Rothwell v Chemical and Insulating Co Ltd and Johnston v NEI International Combustion Ltd* [2007]** discussed further in **section 5.5**.

⁶ Though a claimant may have a claim under one of the intentional torts, e.g. assault or libel or under the tort in *Wilkinson v Downton* [1897] discussed in **Chapter 15**.

⁷ It is crucial then that the elderly woman trapped in a lift (referred to in the examples at the start of this chapter) is able to show that her insomnia and claustrophobia are (or are symptoms of) a recognised psychiatric illness (cf *Reilly v Merseyside RHA* [1995] where the claimants, an elderly couple, were unable to claim for the fear and claustrophobia suffered whilst trapped in a hospital lift for an hour and 20 minutes).

⁸ Imogen Goold and Catherine Kelly 'Who's Afraid of Imaginary Claims? Common Misunderstandings of the Origin of the Action for Pure Psychiatric Injury in Negligence 1888–1943' (2022) 138 Law Quarterly Review 58–78; Joanne Conaghan and Wade Mansell *The Wrongs of Tort* (2nd edn Pluto Press 1999) 36; Jonathan Morgan, *Great Debates in Tort Law* (Hart 2022) ch 9.

⁹ Conaghan and Mansell *ibid*.

¹⁰ Law Commission *Report on Liability for Psychiatric Illness* (Law Com No 249, 1998).

- ¹¹ See also Lord Leggatt and Lady Rose, *Paul* at [47]–[48].
- ¹² Indeed, for many years whiplash claims were big business, with compensation for severe whiplash reaching over £44,000. The government legislated in 2018 to limit such claims. The Civil Liability Act 2018, which came into force in May 2021, introduces (among other things) tariffs reducing the level of compensation for whiplash injuries.
- ¹³ Though secondary victim cases may arise even where there is no primary victim—e.g. they may suffer psychiatric harm as a result of seeing the destruction of *their own* property (see e.g. *Attia v British Gas* [1988] where the claimant was able to recover for the psychiatric harm caused by the destruction of her house caused by the defendant’s negligence while installing central heating).
- ¹⁴ Including *Hicks v Chief Constable of South Yorkshire Police* [1992], *White* and *Airedale NHS Trust v Bland* [1993].
- ¹⁵ As *White* was known in the Court of Appeal. These requirements are set out in full in **section 5.5**.
- ¹⁶ *Yearworth* was not discussed in *Paul*, and so it remains to be seen whether its reasoning will survive the ‘accident’ and ‘incident/crisis’ distinction drawn by the court. See discussion in **section 5.6.3**.
- ¹⁷ The case law here is explored in detail by Rachael Mulheron ‘The “Primary Victim” in Psychiatric Illness Claims: Reworking the “Patchwork Quilt”’ (2008) 19 *Kings Law Journal* 81, 99–106.
- ¹⁸ See e.g. Mulheron *ibid* and Paula Case ‘Now You See It, Now You Don’t: Black Letter Reflections on the Legacies of *White v Chief Constable of South Yorkshire Police*’ (2010) 18 *Tort Law Review* 33.
- ¹⁹ In this case, the court refused to follow the earlier case of *Victorian Railways Commissioners v Coultas* [1888] in which the Privy Council denied the claimant’s claim for psychiatric injury on the basis that her injuries were not reasonably foreseeable and therefore too remote. Their lordships were also concerned that allowing the claim would open the floodgates to speculative or ‘imaginary’ claims (at 226).
- ²⁰ The claimant’s condition was variously described as myalgic encephalomyelitis (ME), or chronic fatigue syndrome (CFS) or post viral fatigue syndrome (PVFS), which had manifested itself from time to time with different degrees of severity.
- ²¹ This extension of the ‘thin’ or ‘egg shell’ skull rule (discussed in more detail in **section 9.3.1.1**), whereby, so long as the relevant kind of harm is foreseeable, it does not matter that its precise form or extent is not, puts the primary victim at a considerable advantage over secondary victims. Unlike secondary victims, primary victims can ‘recover damages for negligently inflicted pure psychiatric illness, even if it was sustained because he lacked the fortitude or “natural phlegm” of an ordinary person’ (Mulheron **n17** 106 arguing for the reintroduction of a rule of normal fortitude in all cases of psychiatric harm).
- ²² See e.g. Stephen Bailey and Donal Nolan’s conclusion to their detailed exploration of *Page* in which they (somewhat optimistically) remark ‘[f]ortunately, the long-term prospects for *Page*’s survival look slim’, and accompanying references (‘The *Page v Smith* Saga: A Tale of Inauspicious Origins and Unintended Consequences’ [2010] *Cambridge Law Journal* 495, 527–8); Lord Goff’s dissenting opinion in *White* (at 468–81) and Peter Handford ‘A New Chapter in the Foresight Saga’ (1996) 4 *Tort Law Review* 5.
- ²³ Though insofar as it removes the requirement that the psychiatric harm be foreseeable, the treatment of primary victims in *Page* is extremely generous.
- ²⁴ On the changing definitions of a primary victim in *Page* [1996], *White* [1998], *W v Essex County Council* [2001] and *Rothwell* [2007], see Mulheron (**n17** 84–6).

²⁵ In February 2010, the government introduced an extra-statutory scheme, which made payments of £5,000 to individuals *who had started but not resolved* a claim for compensation before the decision in **Rothwell**. The payment broadly reflected the level of compensation likely to have been received if pleural plaques had continued to be compensatable ('Pleural plaques' Jack Straw statement, Ministry of Justice, 25 February 2010).

²⁶ Discussed in **section 5.6**.

²⁷ His claim as a rescuer was denied on the basis that he was not physically endangered or reasonably in fear for his own safety (see **section 5.6**).

²⁸ cf *Tredget v Bexley Health Authority* [1994]. This raises an important gender dimension recognised by Philippa Whipple (now Whipple LJ), counsel for the defendant in *Wild v Southend University Hospital NHS Foundation Trust* [2014], whereby fathers are unable to recover in circumstances where mothers are able to recover on virtually the same set of facts. In this case, the claimant was the father of a foetus who died in utero as a result of the defendant's negligence and was later delivered stillborn. He suffered psychiatric harm as a result of being present when the death of the foetus was discovered and during the birth. His claim was rejected on the basis that his witnessing of the consequences of the defendant's negligence did not equate to witnessing a horrific 'event'. While **Paul** removes the requirement of a 'horrific event', it remains to be seen whether a traumatic birth will be considered a medical 'accident' (see discussion in **section 5.6.3.1**).

²⁹ The first 'secondary victim' case was *Hambrook v Stokes Bros* [1925] in which the Court of Appeal refused to follow the bar against recovery in such cases set down by the Divisional Court in *Dulieu v White*.

³⁰ Lord Briggs 'Liability for Mental Injury' Chichester University Lecture, 19 January 2024.

³¹ As noted in **section 5.5**, typically a primary victim does not need to show that their reaction corresponds with that of a person of ordinary fortitude (**Page**) though following *Barber v Somerset County Council* [2004] it seems that this exception does not apply to employees who suffer psychiatric harm due to stress at work (unless the employer knows of a particular problem or vulnerability on the part of the claimant) (see also *Arshad v Wokingham District Council* [2022]) or, following **Rothwell**, to those who suffer an 'unforeseeable' reaction caused by the apprehension of an illness. See further Mulheron **n17** 106–9.

³² It may be that there is greater overlap between the two approaches than this suggests. Lord Bridge made it clear that the three factors set down by Lord Wilberforce would be taken into account when determining whether on the facts the shock was reasonably foreseeable (Donal Nolan '*Alcock v Chief Constable of South Yorkshire Police* (1991)' in Charles Mitchell and Paul Mitchell (eds) *Landmark Cases in the Law of Tort* (Hart 2010) 273, 282). However, as Paula Case notes, the differing approaches of Lords Bridge and Wilberforce are 'broadly representative of an "assimilation approach" which uses ordinary negligence principles to determine duty of care (articulated by Lord Bridge) ... [and an] "isolation approach" [(articulated by Lord Wilberforce)] which views psychiatric damage as distinct from other harms and needing to be insulated from ordinary negligence principles'. She continues: 'Though the latter approach won the day in **Alcock**, more recently case law suggests a movement toward a gradual assimilation of psychiatric harm into mainstream negligence principles in order to determine whether a duty of care exists' (**n18** 34–5). Whether this shift survives **Paul** remains to be seen.

³³ See further **sections 3.2** and **7.4**.

³⁴ In fact, the Hillsborough Stadium disaster was one of a number of man-made disasters during the UK's 'disaster era' (1985–9), which included the Bradford football stadium fire in 1985 (40 fatalities), the sinking of the *Herald of Free Enterprise* car ferry (187) and the King's Cross Underground station fire in 1987 (31), the Piper Alpha oil rig fire in 1988 (167) and, in 1989, the East Midlands Kegworth plane crash (47) and the sinking of the *Marchioness* pleasure boat (51). Nolan suggests that the fact that there were so many man-made disasters in the years preceding **Alcock**—he lists ten events, totalling 979 fatalities, each giving rise to litigation (including for psychiatric injury)—combined with the 'topicality of trauma-induced psychiatric injury ... can only have reinforced long-standing judicial concerns about the opening of floodgates in nervous shock litigation' (n22 292).

³⁵ Phil Scraton 'Justice: Hillsborough's Final Victim' (1992) April, Legal Action 7. The legacy of Hillsborough continues (see BBC News 'Hillsborough: Timeline of the 1989 stadium disaster' 8 April 2022). In April 2016, after almost 27 years of campaigning by friends and family members of those who died at Hillsborough, an inquest ruled that they were unlawfully killed and a catalogue of failings by police and the ambulance services contributed to their deaths (David Conn 'Hillsborough inquests jury rules 96 victims were unlawfully killed' *The Guardian* 26 April 2016). In June 2021, it was reported that South Yorkshire and West Midlands police had agreed to pay compensation (arising out of a claim in the tort of misfeasance in public office) to over 600 survivors, and friends and family members of people who had died at Hillsborough, for the additional trauma and psychiatric damage caused by the police response to the disaster (David Conn 'South Yorkshire and West Midlands police agree payouts for Hillsborough "cover-up"' *The Guardian* More on the Hillsborough disaster can be found in this chapter's web links <<https://iws.oup.support.com/ebook/access/content/horse9e-student-resources/horse9e-chapter-5-web-links?options=showName>> 4 June 2021). Despite multiple civil and criminal claims, no one has been held to account for the tragedy at Hillsborough; a fact described by Labour MP Maria Eagle as a 'catastrophic failure of justice' (BBC News online, 'Lack of Hillsborough accountability is a scandal, says minister' 27 May 2021).

³⁶ It was accepted, for the purposes of argument, that the claimants were suffering from a recognised psychiatric illness and that the chief constable had breached his duty of care and had caused the claimants' injuries. The question of breach, in light of the Hillsborough Inquest's findings in 2016 that the failure of David Duckenfield, the Hillsborough police match commander, to close a tunnel 'was the direct cause of the deaths of 96 people,' now seems unarguable (BBC News 'Hillsborough inquests: David Duckenfield admits causing 96 deaths' 17 March 2015).

³⁷ See e.g. the reaction of Trevor Hicks who was at the stadium during the disaster with his daughters (both of whom died). ('Families' fury at cash bid by Hillsbro cops' *Daily Mirror* 1 November 1996).

³⁸ Conaghan and Mansell n8 40.

³⁹ *Report on Liability for Psychiatric Illness* n10 [6.10].

⁴⁰ This seems at odds with the decisions in *Walker* (discussed in **section 5.4**). Lord Hoffmann, in the majority, distinguished *Walker* in **White** on the somewhat dubious basis that in *Walker* the claimant's psychiatric harm 'was caused by the strain of doing the work which his employer had required him to do' (at 506), whereas in **White** the police officers' injuries stemmed from their *witnessing* of the death and injury of others. Lord Griffiths would have allowed the appeal on the basis of the police officers' position as employees. In his dissent, he rejected the distinction between physical and psychiatric injury; '[i]f it is foreseeable that the rescuer may suffer personal injury in the form of psychiatric injury rather than physical injury, why should he not recover for that injury?' (at 464). See further Paula Case 'Now You See It, Now You Don't: Black Letter Reflections on the Legacies of *White v Chief Constable of South Yorkshire Police*' (2010) 18 Tort Law Review 33, 38.

⁴¹ In the same vein, rescuers' actions will usually not be treated as *novus actus interveniens* nor will they be held to be contributorily negligent in respect of any harms they suffer in attempting their rescue.

⁴² An alternative way to distinguish *Chadwick* and **White** would have been to introduce the so-called 'fireman's rule', whereby the police officers in **White** as professional rescuers are considered to be persons 'of extraordinary phlegm' and recovery is restricted accordingly. This solution was rejected by the majority of the Court of Appeal and by Lord Hoffmann in **White**.

⁴³ Jane Stapleton 'In Restraint of Tort' in Peter Birks (ed) *The Frontiers of Liability: Volume 2* (OUP 1994) 95.

⁴⁴ *ibid*.

⁴⁵ *Report on Liability for Psychiatric Illness* n10 [6.3]. Lord Burrows was the Law Commissioner who led the project. The Report concluded that wholesale legislative reform of the area was unnecessary, but:

that in some respects, and most notably in the decision of the House of Lords in **Alcock v Chief Constable of South Yorkshire Police**, the common law has taken a wrong turn. Legislation can cure the defects in the common law at a stroke and with certainty. To wait for the House of Lords to reverse **Alcock** may be to wait for a very long time indeed. Our policy may therefore be described as one of recommending minimal legislative intervention curing serious defects in the present law but otherwise leaving the common law to develop ... we recommend that, while legislation curing serious defects in the present law is appropriate, the law should otherwise be allowed to develop by judicial decision-making. (at [4.2])

⁴⁶ At [23].

⁴⁷ Stapleton n43 95.

⁴⁸ Nolan n32 308.

⁴⁹ Lord Ackner in **Alcock** suggested that witnessing an out-of-control petrol tanker crash into a school may be such an event (at 403).

⁵⁰ Though the boat he was on had come within 80 metres of the oil rig, his fear for his own life was not reasonable. This aspect of the case should now be read in light of **Page**.

⁵¹ The bomb, planted by the IRA, was detonated as members of the Household Cavalry rode past from Knightsbridge Barracks to Horse Guards Parade for the Changing of the Guard. Four soldiers, including the claimant's father, Lance Corporal Young, were killed by the bomb, 31 other people were injured and seven horses had to be destroyed (BBC News '1982: IRA bombs cause carnage in London' On this day, 20 July 1982). Downey had been found to be liable for the attack in civil law in an earlier hearing (*Young v Downey* [2019]; BBC News 'John Downey "active participant" in Hyde Park bombing' 18 December 2019).

⁵² We agree with David Knifton that 'whilst it is difficult to fault the logic of Spencer J's approach to the law, it is somewhat troubling that, having specifically invited oral evidence from a very experienced psychiatric expert in order to elucidate what might have been in the mind of a 4-year-old child, the judge then chose to reject that evidence, and to substitute his own view' ('Another Hurdle for Nervous Shock Claims' *Exchange Chambers blog* 5 February 2021).

⁵³ Her dependency claim under the Fatal Accidents Act 1976 was successful. She was awarded £713,457, apportioned as £178,364 to the claimant and £535,093 to the claimant's mother.

⁵⁴ See also Tomlinson LJ's rationalisation of **McLoughlin** where he suggested that while it is usually understood as an 'aftermath' case, 'it could properly be said that Mrs McLoughlin came upon the accident, albeit transposed into the setting of the hospital' (*Ronayne* at [16]).

⁵⁵ Stapleton **n43** 84.

⁵⁶ Counsel in *Berisha v Stone Superstore Ltd* [2014] (in which the claimant's claim as a secondary victim was struck out: her arrival at the hospital 5 hours after her partner had been seriously injured at work and staying with him for a further 36 hours until his life support was removed, was not part of the accident's 'immediate aftermath') suggested that **McLoughlin** and *Galli-Atkinson* are the *only* reported examples of a claimant who attends hospital after an unexpected accident has befallen a loved one successfully recovering damages for psychiatric harm (at [35]).

⁵⁷ cf Lord Burrows (in dissent), arguing that the Court of Appeal's decision in **Novo** should be overruled, with Lord Dyson, who considered that:

the ordinary reasonable person would find it unreasonable and incomprehensible that a person who suffered psychiatric illness by coming too late to an accident causing injury or death could not recover, while a person who witnessed the later death from the accident could recover. However, there is a clear distinction between the two situations because, if one treats the accident and the death as separate events, the person who can recover is the same person in both situations ie the person with a close tie of love and affection, provided that that person witnesses (and is therefore closely proximate to) either the accident or the death. One is therefore treating the two situations in the same way and there is nothing unacceptable or irrational about so doing. In any event, applying the same hypothetical logic as Lord Dyson, I would suggest that the ordinary reasonable person would find it unacceptable and incomprehensible that, varying the facts of **Novo**, a daughter who witnessed the initial injury to the primary victim and suffered a psychiatric illness could recover whereas a daughter who witnessed the horrific death of the primary victim a few weeks later and suffered a psychiatric illness could not recover. (at [238])

⁵⁸ In **Alcock**, none of the live television coverage, in line with broadcasting policy, showed pictures of suffering by recognisable individuals. Had it done so, it was accepted that this would act as a *novus actus interveniens* breaking the chain of causation between the defendant's alleged breach of duty and the psychiatric illness (at 410).

⁵⁹ Though see also *Tredget v Bexley Health Authority* [1994] and *RE (A minor by her mother and Litigation Friend LE) and others v Calderdale & Huddersfield NHS Foundation Trust* [2017] (both of which allowed a secondary victim (a father and a grandmother respectively) to recover following them witnessing a particularly traumatic birth).

⁶⁰ Despite, as noted by Lord Burrows, Lord Rodger in *D v East Berkshire Community Health NHS Trust* [2005] relying on *Walters* to support his view that 'medical mishaps' fell within the category of secondary victims (**Paul** at [126]).

⁶¹ Jonathan Morgan suggests that this reliance on the rule in *Baker v Bolton* [1808] is 'questionable' ('Principles, Accidents and Exceptions: Psychiatric Injury in Tort' [2004] *Cambridge Law Journal* 214, 215–16).

⁶² While Lord Leggatt and Lady Rose define an accident as an event external to the *primary* victim, Lord Burrows suggests this 'may be regarded as drawing a somewhat arbitrary line': 'one might alternatively look at the matter from the perspective of the secondary victim and define an accident as an event external to the secondary victim. Defined

in that way, the death of the primary victim is an accident because it is an event external to the secondary victim. ... what is the justification for adopting that definition of an accident and not another?; or, put another way, why do some accidents count and others do not?' (at [211]).

⁶³ James Lee 'Incidents and Accidents—The Supreme Court on Psychiatric Harm' Inner Temple Lecture, 6 March 2024: www.youtube.com/watch?v=4t_W3LCWzHM <https://www.youtube.com/watch?v=4t_W3LCWzHM>. The decision in *RE* may also be on point here. You may like to listen to this excellent podcast episode (Kings Chambers Debrief S2 E1: 'Secondary victims in clinical negligence: thus far and absolutely no further' <<https://www.buzzsprout.com/179951/episodes/14296978-kings-chambers-debrief-s2-e1-secondary-victims-in-clinical-negligence-thus-far-and-absolutely-no-further>>) produced by Kings Chambers which features a conversation between Helen Mulholland KC and Victoria Heyworth, where they discuss *Paul* and the likely effect of it on secondary victim claims in cases of clinical negligence. The podcast is just over 37 minutes long so if you don't have time to listen to it right now, just make a note to return to it later—or perhaps download it to listen to on the way to your tort seminar!

⁶⁴ Happily, after nine weeks in intensive care, during which time she developed an MRSA infection and had to deal with other extremely unpleasant complications, the claimant's wife made a full recovery.

⁶⁵ Morgan **n61** 214.

⁶⁶ Jamie Lee **n63**.

⁶⁷ Morgan **n61** 216.

⁶⁸ NHS Resolution 'Annual Report Statistics 2023–2024' 27 September 2024.

⁶⁹ NHS Resolution 'Case of note: *Paul v Royal Wolverhampton NHS Trust; Polmear v Royal Cornwall Hospitals NHS Trust; Purchase v Dr Ahmed* (Supreme Court—11 January 2024)' 16 February 2024.

⁷⁰ Ministry of Justice *The Law on Damages, Response to Consultation* (CR[®] 9/07, 2009) 51.

⁷¹ Lord Briggs **n30** [58].

© Kirsty Horsey & Erika Rackley 2025

Related Books

View the Essential Cases in tort law

Related Links

Test yourself: Multiple choice questions with instant feedback <<https://learninglink.oup.com/access/content/brennan-concentrate6e-student-resources/brennan-concentrate6e-diagnostic-test>>

Find This Title

In the OUP print catalogue <<https://global.oup.com/academic/product/9780198925231>>